

The Geometry of the Cure

Geometric Therapeutic Optimization via the Davis Field Equations

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Abstract

Compartmentalized infections—those in which anatomical, biophysical, or phenotypic barriers separate the pathogen from the vascular compartment—account for a disproportionate share of treatment failures in infectious disease. Standard pharmacokinetic reasoning, which equates serum drug concentrations with tissue drug concentrations, fails predictably and repeatedly in bone, lung granuloma, cerebrospinal fluid, and latent viral reservoirs.

We introduce MIRADOR (*Manifold-Informed Rational Architecture for Drug-Organism Response*), a geometric framework for predicting drug efficacy in compartmentalized infections, requiring zero pharmacokinetic fitted parameters. The framework rests on a single governing equation: therapeutic coherence $C = \tau/K$, where $\tau = \log_{10}(\text{AUC}_{24}/\text{MIC})$ is the pharmacophoric potential of a drug and K is the total pathway impedance—a series sum of barrier, phenotype, reservoir, and systemic curvature terms, each computed entirely from published pharmacokinetic data. The only calibrated quantity is a single disease-specific diagnostic threshold that translates the continuous geometric ranking into a binary clinical verdict; the impedances, potentials, and drug rankings themselves are parameter-free. Multi-drug regimens are combined via a Kirchhoff parallel-resistor law on drug conductances, and the framework’s predictive boundary is formally characterized by the Double Cover Identity ($S + d^2 = 1$), which partitions every therapeutic problem into what penetration geometry explains and what it cannot.

We validate MIRADOR across four disease instances—pediatric bone MRSA (osteomyelitis), pulmonary tuberculosis, bacterial meningitis, and HIV latent reservoirs—spanning four pathogens, four organ systems, and four barrier types, using the same equation throughout. Across four disease instances, with strict separation of pharmacokinetic inputs from clinical ground truths, the framework retrospectively reproduces established drug rankings, matches documented clinical phenomena not used in model construction, and identifies five novel predictions including the geometric curability of the HIV genital tract reservoir and CSF viral escape as a geometric inevitability; for tuberculosis, computed lesion coherence additionally correlates with clinical week-8 culture conversion ($r = 0.81$, $n = 6188$ across 15 trial arms). All code is implemented in Rust with 290 passing tests across 29 crates.

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Patent notice. Subject matter described in this manuscript may relate to US Provisional Patent Application No. 64/012,328: “MIRADOR: Manifold-Informed Rational Architecture for Drug-Organism Response.” Commercial use of the methods described herein may require a licence.

1 Introduction

1.1 The compartment problem

The pharmacokinetic foundation of modern antibiotic therapy rests on a single implicit assumption: that the drug concentration at the site of infection is well-approximated by the drug concentration in serum. This assumption is encoded in every dosing guideline that targets a serum trough, every PK/PD index computed from plasma AUC, and every susceptibility breakpoint calibrated against blood-level data. When it holds, it is extraordinarily useful.

When it fails, patients die.

The assumption fails whenever an anatomical, biophysical, or phenotypic barrier interposes between the vascular compartment and the pathogen population. We call such infections *compartmentalized*, and formalize the failure as a single inequality:

$$C_{\text{site}} \neq C_{\text{serum}}. \tag{1}$$

This inequality is not new. Its consequences have been documented, disease by disease, for decades:

- **Bone.** Vancomycin—the standard of care for MRSA osteomyelitis—achieves bone-to-serum AUC ratios of 0.20–0.37 in infected tissue, with penetration declining as osteomyelitis progresses [1, 2, 3]. A patient whose serum vancomycin meets the recommended AUC/MIC ≥ 400 target may have a bone AUC/MIC below the minimum biofilm eradication threshold by an order of magnitude.
- **Lung granuloma.** In pulmonary tuberculosis, the avascular caseous core of granulomas acts as a diffusion barrier whose permeability is drug-specific and heterogeneous. Caseum binding inversely correlates with passive diffusion into the necrotic core; for several first-line agents, caseum penetration is near zero [4, 5, 6]. The bacteria that survive treatment are overwhelmingly those harbored in caseum.
- **Cerebrospinal fluid.** The blood-brain barrier excludes the vast majority of systemically administered drugs from the CNS. CSF-to-plasma ratios for many antibiotics and antiretrovirals fall below 0.05, and even drugs classified as “CNS-penetrating” rarely exceed 0.30 [7, 8].
- **HIV latent reservoirs.** Antiretroviral therapy suppresses HIV to undetectable plasma levels, but provirus integrated into the host genome persists in anatomically sequestered reservoirs—gut-associated lymphoid tissue (GALT), CNS, lymph nodes, bone marrow, and genital tract—each with distinct tissue-to-plasma ratios and an additional phenotypic barrier: the latent provirus is invisible to drugs that require active viral replication [9, 10, 11].

The clinical consequence is the same in each case: physicians make dosing decisions on the basis of serum concentrations, and treatment fails in exactly the compartments where barriers are highest. The drugs exist. The biology is understood. What is missing is the mathematics to deploy existing drugs to the places where disease hides.

1.2 The gap

Existing pharmacokinetic modeling frameworks address the compartment problem at high parametric cost. Physiologically-based pharmacokinetic (PBPK) models simulate drug disposition across organ compartments using systems of differential equations whose parameters—organ volumes, blood

flows, partition coefficients, metabolic rates, transporter kinetics—number in the dozens to hundreds per drug-tissue combination [12, 13, 14]. Population PK models require large clinical datasets for each drug in each compartment. Both approaches predict absolute concentrations, and both are indispensable: the tissue-to-plasma ratios and AUC data that any higher-level framework requires are produced by exactly this empirical and computational pharmacology. The gap is not in the data these models generate, but in the absence of a unifying geometric layer that consumes their outputs across diseases with a single equation, requires no additional fitted parameters, and formally characterizes its own predictive boundary.

We can state the gap precisely. Let $N_{\text{params}}(M)$ denote the number of fitted parameters in a model M that predicts relative drug efficacy rankings at a compartmentalized infection site. For current approaches:

$$N_{\text{params}}(\text{PBPK}) \gg 0, \quad N_{\text{params}}(\text{PopPK}) \gg 0. \quad (2)$$

No existing framework satisfies all of the following simultaneously: (i) $N_{\text{params}}^{\text{PK}} = 0$ (zero pharmacokinetic fitted parameters—all impedances and potentials computed from published data, with only a single disease-specific diagnostic threshold calibrated against an established standard of care); (ii) inputs consist solely of published pharmacokinetic data (tissue:plasma ratios, AUC, MIC); (iii) a single equation applies across disease instances; (iv) the output is a drug ranking (not an absolute concentration); and (v) the framework provides a formal, quantitative characterization of its own predictive boundary.

1.3 Contribution

This paper introduces MIRADOR, a geometric framework for therapeutic optimization in compartmentalized infections. The framework is built on three equations that we preview here and derive formally in Section 2.

Equation 1: Therapeutic coherence. For a single drug d at a single reservoir r :

$$C(d, r) = \frac{\tau(d)}{K_{\text{pathway}}(d, r)} \quad (3)$$

where $\tau(d) = \log_{10}(\text{AUC}_{24}(d)/\text{MIC}(d))$ is the *pharmacophoric potential*—the drug’s intrinsic killing power in the absence of barriers—and K_{pathway} is the *total pathway impedance*, a series sum of curvature terms encoding every obstacle between the vasculature and the pathogen. Higher C means fewer barriers between drug and disease. The equation is a special case of the Davis Field Equations ($C = \tau/K$) applied to the therapeutic manifold.

Equation 2: Kirchhoff combination law. For a cocktail of N drugs at reservoir r , total coherence is the sum of individual drug coherences (coupled mode, parallel-resistor topology):

$$C_{\text{combo}}(r) = \sum_{i=1}^N \tau_i g_i, \quad g_i = \frac{1}{K_{\text{pathway},i}}. \quad (4)$$

Each drug contributes conductance $g_i = 1/K_{\text{pathway},i}$; drugs with high impedance contribute near-zero conductance even in combination. A decoupled variant for drugs targeting independent pathogen subpopulations is presented in Section 2.3.

Equation 3: The Double Cover Identity.

$$\boxed{S + d^2 = 1} \tag{5}$$

where S is the fraction of the therapeutic problem explained by penetration geometry (drug distribution through barriers) and d^2 is the fraction explained by non-geometric dynamics (kill kinetics, immune response, reactivation biology). Formally, for an infection spanning N anatomical reservoirs, $S = |\{r : C_{\text{combo}}^{\text{active}}(r) \geq \theta\}|/N$, the fraction of reservoirs at which the geometric combination coherence meets or exceeds the therapeutic threshold θ . This is the framework’s built-in uncertainty quantification: when $S \approx 1$, trust the drug ranking; when $S \approx 0$, the model has detected its own boundary.

MIRADOR is validated across four disease instances: pediatric bone MRSA (osteomyelitis), pulmonary tuberculosis, bacterial meningitis, and HIV latent reservoirs. The mathematical structure— $C = \tau/K$ with disease-specific K content—is identical in each case. Across the four disease instances, with strict separation of pharmacokinetic inputs from clinical ground truths and zero pharmacokinetic fitted parameters, the framework:

1. reproduces clinically established drug rankings in bone MRSA, TB, and meningitis;
2. predicts five documented clinical phenomena not used in model construction (CSF viral escape, vancomycin bone failure, rifampin’s privileged intracellular role, the TB rank inversion, and PI preference for CNS-targeted HIV regimens);
3. identifies five novel predictions, including the geometric curability of the HIV genital tract reservoir with existing agents and the quantitative shortfall to functional cure at GALT; and
4. formally detects its own predictive boundary in the TB instance, where the geometric drug ranking inverts the clinical sterilization ranking—an inversion the Double Cover identifies as the transition from geometry-dominated ($S = 0.78$) to dynamics-dominated ($d^2 = 0.22$) behavior.

1.4 Paper organization

Section 2 presents the mathematical framework: definitions, the coherence equation, the combination law, the No Parallel Lines axiom, catalytic state modification, and the Double Cover Identity. Sections 3 to 6 present the four disease instances with their disease-specific K structures and validation results. Section 7 contains the cross-disease analysis and universality claim. Section 8 positions MIRADOR relative to PBPK models, PK/PD indices, CPE scores, and information geometry. Section 9 describes the validation philosophy (circular logic firewall, zero-parameter claim). Section 10 provides honest accounting of limitations, formalized as mathematical omissions. Section 11 states clinical implications as corollaries of theorems. Section 12 outlines future work. Section 13 closes on the equation.

2 Mathematical Framework

This section presents the complete mathematical framework. Every quantity is defined from published pharmacokinetic data; the only calibrated quantity is a single disease-specific diagnostic threshold θ (Section 1). We state definitions and key results; the companion monograph provides full derivations and geometric intuition for each choice.

2.1 Geometric foundations

The mathematical framework of MIRADOR rests on two foundational results from the Davis geometric program, both developed by Davis [35, 36] as part of a research program applying Riemannian and fiber-bundle geometry to phase transitions across physical domains.

The Double Cover Principle. Davis [35] proves that the unit circle S^1 carries two distinct geometric structures: the *holonomy space* S^1 with circumference $\tau = 2\pi$, governing rotations and geometric phase, and the *sameness space* $\mathbb{RP}^1$ with circumference π , governing identity decay and state overlap. These spaces are related by the squaring map $q: S^1 \rightarrow \mathbb{RP}^1$ of covering degree 2, and the fundamental identity

$$S + d^2 = 1, \tag{6}$$

where $S = \cos^2(\theta/2)$ is the sameness (how much identity a rotated state retains) and $d = \sin(\theta/2) = K/2$ is the normalized chord distance, is the Pythagorean theorem on this double cover. The Davis capacity $C = \tau/K$ then has the transparent geometric meaning $C = \pi/\sqrt{1-S}$: a system undergoes a phase transition exactly when the curvature cost has consumed the entire sameness budget, yielding the critical capacity $C_{\text{crit}} = \pi$ —the circumference of the sameness space.

In the pharmacokinetic setting of this paper, the Double Cover Principle becomes the Double Cover Identity (Theorem 2.28): S is the geometric coverage (fraction of reservoirs reached above threshold) and $d^2 = 1 - S$ is the fraction where unmodeled dynamics dominate. The partition is not a modeling convenience but a structural consequence of the two-space geometry.

The Non-Decoupling Theorem. Davis [36] proves that a system admitting a faithful description as a principal fiber bundle with connection ∇ can be modeled by decoupled (connection-free) components if and only if the curvature 2-form Ω vanishes identically. When curvature is nonzero, any flat model incurs irreducible structural error bounded below by the Yang–Mills functional $\|\Omega\|_{L^2}^2$, and when the bundle carries non-trivial characteristic classes, no flat connection exists at all—decoupled models are categorically impossible.

The core insight is that the decoupling assumption—that a system can be decomposed into independent components whose outputs are later combined—is the parallel postulate applied to state space. In the pharmacokinetic setting, this becomes the No Parallel Lines axiom (Assumption 2.19): the therapeutic manifold has everywhere-nonzero, everywhere-finite curvature. No pathway is perfectly unimpeded ($K = 0$) and no pathway is perfectly blocked ($K = \infty$), because the underlying geometry is non-Euclidean.

2.2 Definitions

Definition 2.1 (Compartmentalized infection). An infection in which the drug concentration at the site of pathogen residence differs from the drug concentration in systemic circulation due to one or more anatomical, biophysical, or phenotypic barriers between the vascular compartment and the pathogen population. Formally, an infection is compartmentalized if there exists a drug d and a pathogen reservoir r such that $C_{\text{site}}(d, r) \neq C_{\text{serum}}(d)$.

Definition 2.2 (Pharmacophoric potential). For a drug d , the *pharmacophoric potential* is

$$\tau(d) = \log_{10} \left(\frac{\text{AUC}_{24}(d)}{\text{MIC}(d)} \right), \quad (7)$$

where $\text{AUC}_{24}(d)$ is the area under the plasma concentration–time curve over 24 hours at standard dosing, and $\text{MIC}(d)$ is the minimum inhibitory concentration against the target pathogen. For viral pathogens, MIC is replaced by IC_{50} ; the functional role is identical. The quantity τ encodes the intrinsic potency of drug d in the absence of all barriers: the ceiling of what the drug can achieve if the pathogen resided in plasma.

Remark 2.3 (Why $\log_{10}$). The choice of $\log_{10}$ (rather than the natural logarithm) is deliberate: AUC/MIC ratios in clinical pharmacology span 10^0 – 10^3 , and the $\log_{10}$ transform maps this range to $[0, 3]$, aligning τ with the order-of-magnitude language used in antibiotic dosing (“the AUC/MIC is 400” corresponds to $\tau \approx 2.6$). No mathematical property of the framework depends on the base of the logarithm; all rankings are preserved under any monotone transformation.

Definition 2.4 (Barrier curvature). For a drug d at reservoir r with tissue-to-plasma concentration ratio $R(d, r) > 0$, the *barrier curvature* is

$$K_{\text{barrier}}(d, r) = \max \left(\frac{1}{R(d, r)} - 1, -1 \right). \quad (8)$$

The three regimes are:

- $R < 1$ (drug excluded): $K_{\text{barrier}} > 0$ (positive curvature, impedance). Example: vancomycin in bone, $R \approx 0.20$, $K_{\text{barrier}} = 4.0$.
- $R = 1$ (no barrier): $K_{\text{barrier}} = 0$ (flat geometry).
- $R > 1$ (drug concentrates): $K_{\text{barrier}} \in [-1, 0)$ (negative curvature, facilitation). The floor at -1 is the No Parallel Lines axiom (Section 2.4).

Definition 2.5 (Pathway impedance). For a drug d at reservoir r , the *total pathway impedance* is the series sum

$$K_{\text{pathway}}(d, r) = K_{\text{ADMET}}(d) + K_{\text{barrier}}(d, r) + K_{\text{phenotype}}(d, r) + K_{\text{reservoir}}(r), \quad (9)$$

where:

- $K_{\text{ADMET}}(d) \geq 0$ is the systemic ADMET (absorption, distribution, metabolism, excretion, toxicity) penalty;
- $K_{\text{barrier}}(d, r)$ is the barrier curvature (Definition 2.4);

- $K_{\text{phenotype}}(d, r) \geq 0$ is the phenotypic resistance of the pathogen at reservoir r to drug d (biofilm, dormancy, latency);
- $K_{\text{reservoir}}(r) \geq 0$ is the anatomical niche persistence of reservoir r (independent of drug).

Each term represents a distinct physical obstacle between the vasculature and the pathogen. The series structure encodes the assumption that obstacles compose additively: a drug must traverse all layers, and each layer contributes independently to the total impedance.

Definition 2.6 (Therapeutic coherence). For a single drug d at a single reservoir r with $K_{\text{pathway}}(d, r) > 0$, the *therapeutic coherence* is

$$C(d, r) = \frac{\tau(d)}{K_{\text{pathway}}(d, r)} \quad (10)$$

This is the governing equation of the framework. C is dimensionless and represents the ratio of intrinsic potency to total delivery impedance. A drug with $C \geq \theta$ (the disease-specific diagnostic threshold) can suppress the pathogen at reservoir r ; a drug with $C < \theta$ fails at that site regardless of serum-level adequacy.

Remark 2.7 (Relationship to the Davis Field Equations). Equation (10) is a domain-specific instantiation of the Davis Field Equations, $C = \tau/K$, in which the abstract tolerance τ is realized as pharmacophoric potential and the abstract curvature K is realized as pathway impedance computed from published pharmacokinetic data. The framework inherits the general properties of Davis systems—compositional error budgets, bounded geodesic–Euclidean distortion, and the Double Cover Principle [35]—by construction. The Non-Decoupling Theorem [36] provides the formal justification for why the curvature terms K cannot be neglected: any flat approximation to the pathway geometry incurs irreducible error proportional to the curvature magnitude. The companion monograph [37] provides full derivations and geometric intuition for each choice.

Remark 2.8 (Open-source implementation). The governing equation is implemented in the open-source MIRADOR engine (Rust) as `coherence(tau, k)` in `mirador-core/src/coherence.rs`, with 11 unit tests verifying linearity in τ , inverse proportionality in K , and the Double Cover identity. The full codebase is available at <https://github.com/nurdymuny/mirador>.

2.3 Combination law

When multiple drugs are administered simultaneously at a given reservoir, their individual conductances compose in parallel, exactly as resistors in a Kirchhoff circuit. However, the correct combination formula depends on whether the drugs target the *same* pathogen population or *independent* subpopulations.

Definition 2.9 (Drug conductance). For a drug d at reservoir r with $K_{\text{pathway}}(d, r) > 0$, the *conductance* is $g(d, r) = 1/K_{\text{pathway}}(d, r)$.

Definition 2.10 (Combination coherence — coupled mode). When N conductive drugs $\{d_1, \dots, d_N\}$ target the **same pathogen population** at reservoir r (shared-pathway drugs), the combination

coherence is:

$$\tau_{\text{combo}}^c = \frac{\sum_{i=1}^N \tau_i g_i}{\sum_{i=1}^N g_i}, \quad (11)$$

$$K_{\text{combo}}^c = \frac{1}{\sum_{i=1}^N g_i}, \quad (12)$$

$$C_{\text{combo}}^c(r) = \frac{\tau_{\text{combo}}^c}{K_{\text{combo}}^c} = \sum_{i=1}^N \tau_i g_i, \quad (13)$$

where $g_i = g(d_i, r)$ and $\tau_i = \tau(d_i)$. This is the *paired-sum* (Kirchhoff) formula: a drug's contribution to the cocktail is weighted by its own conductance at the reservoir. A drug with high intrinsic potency but negligible penetration contributes negligibly.

Definition 2.11 (Combination coherence — decoupled mode). When N conductive drugs target **independent pathogen subpopulations** at reservoir r (e.g., Mitchison subpopulations in tuberculosis, where each drug clears a distinct phenotypic niche), the combination coherence is:

$$C_{\text{combo}}^d(r) = \left(\sum_{i=1}^N \tau_i \right) \left(\sum_{i=1}^N g_i \right). \quad (14)$$

This is the *decoupled-product* formula: τ contributions are additive because each drug addresses a distinct target population, and total conductance sums independently.

The relationship between the two modes is a strict inequality for any nontrivial cocktail.

Proposition 2.12 (Cross-term ordering). *For any cocktail of $N \geq 2$ drugs with $\tau_i, g_i > 0$:*

$$C_{\text{combo}}^c(r) = \sum_i \tau_i g_i < \left(\sum_i \tau_i \right) \left(\sum_i g_i \right) = C_{\text{combo}}^d(r). \quad (15)$$

The inequality is strict whenever $N \geq 2$. For $N = 1$, the two modes coincide: $\tau_1 g_1 = \tau_1 g_1$.

Proof. Expanding the right-hand side:

$$\left(\sum_i \tau_i \right) \left(\sum_j g_j \right) = \sum_i \tau_i g_i + \sum_{i \neq j} \tau_i g_j.$$

The cross-term $\sum_{i \neq j} \tau_i g_j > 0$ since every $\tau_i, g_j > 0$. Therefore the decoupled product strictly exceeds the coupled sum for $N \geq 2$. $\square$

Remark 2.13 (When to use which mode). The choice of coupling mode is not a free parameter—it is determined by the biology of the disease instance:

- **Coupled mode** (default): all drugs compete for access to the same pathogen phenotype at the same anatomical site. Used for HIV reservoirs (all ARVs target actively replicating virus), meningitis (all antibiotics target planktonic bacteria in CSF), and bone MRSA (all antibiotics target the same biofilm population).
- **Decoupled mode**: drugs clear phenotypically distinct subpopulations. Used for pulmonary TB, where INH targets actively replicating bacilli, RIF targets semi-dormant persisters, PZA targets acid-phase dormant bacilli, and EMB targets intracellular organisms—each in a distinct Mitchison niche with distinct $K_{\text{phenotype}}$ values.

The decoupled mode is systematically more optimistic (Proposition 2.12), which is physically correct: when drugs address independent subpopulations, the total killing power is greater than when they compete for the same pathway.

Remark 2.14 (Why isolate τ_{combo} and K_{combo}). In coupled mode, $C_{\text{combo}}^c = \sum \tau_i g_i = \sum C_i$ —the combination coherence is the sum of individual coherences. A reader may ask why the intermediate quantities τ_{combo} and K_{combo} are defined at all. Two reasons: (i) when catalytic modification enters (Section 2.5), the total coherence is $C_{\text{total}} = f'_s \times C_{\text{combo}}$, and the catalytic prefactor multiplies the *system-level* coherence, not individual drug coherences; (ii) the system impedance $K_{\text{combo}} = 1/\sum g_i$ is the clinically interpretable quantity for predicting sensitivity to global barrier fluctuations (e.g., how much does C_{combo} change if inflammation alters all R values by a common factor?).

Remark 2.15. In the Introduction (Eq. (4)), a synergy scalar appeared in the combination formula. We use synergy = 1.0 (strict additivity) for meningitis, TB, and HIV; for bone MRSA, the beta-lactam + rifampin combination uses $s = 1.2$ based on published synergy data [20]. The structural slot is preserved for future extensions incorporating experimentally measured drug–drug interactions. In decoupled mode with synergy = s , the combination coherence becomes $s^2 \cdot (\sum \tau_i)(\sum g_i)$; the squared entry reflects that synergy scales both the potency pool and the conductance pool independently.

Three properties of the combination law hold in both coupling modes and are important.

Proposition 2.16 (Monotonicity). *Adding a drug to a cocktail can never decrease C_{combo} in either mode. In coupled mode, $C_{\text{combo}}^{c,(N+1)} = C_{\text{combo}}^{c,(N)} + \tau_{N+1}g_{N+1} > C_{\text{combo}}^{c,(N)}$. In decoupled mode, both factors $\sum \tau_i$ and $\sum g_i$ increase.*

Proof. Coupled: immediate from $\tau_{N+1}g_{N+1} > 0$. Decoupled: $(\sum_{i=1}^{N+1} \tau_i)(\sum_{i=1}^{N+1} g_i) > (\sum_{i=1}^N \tau_i)(\sum_{i=1}^N g_i)$ since each factor strictly increases. $\square$

Proposition 2.17 (τ_{combo} bounds (coupled mode)). *In coupled mode, the combination pharmacophoric potential lies between the component extremes: $\min_i(\tau_i) \leq \tau_{\text{combo}}^c \leq \max_i(\tau_i)$.*

Proof. τ_{combo}^c is a convex combination of the τ_i (with weights $g_i/\sum_j g_j > 0$, summing to 1). $\square$

Proposition 2.18 (Dominance by high-impedance drugs). *A drug with $K_{\text{pathway},i} \rightarrow \infty$ contributes $g_i \rightarrow 0$. In coupled mode, its contribution $\tau_i g_i \rightarrow 0$ (effectively absent). In decoupled mode, its conductance contribution vanishes but its τ_i still enters the potency sum—however, the product $(\sum \tau_i)(\sum g_i)$ is dominated by drugs with finite g_i , so the net contribution is negligible for practical cocktail sizes.*

This is the mathematical content of the clinical observation that “vancomycin doesn’t work in bone even in combination”: its bone impedance $K_{\text{pathway}} \approx 13$ yields a conductance $g \approx 0.08$, which is dominated by drugs with $K_{\text{pathway}} \approx 3\text{--}5$ ($g \approx 0.2\text{--}0.3$).

2.4 The No Parallel Lines axiom

The following axiom is the pharmacokinetic instantiation of the Davis Non-Decoupling Theorem [36], which establishes that systems with nonzero curvature cannot be faithfully modeled by decoupled components (Section 2.1).

Assumption 2.19 (No Parallel Lines). Every curvature term in the framework is finite for drugs that retain their mechanism of action. Specifically:

- (i) $K_{\text{barrier}}(d, r) \leq K_{\text{max}} < \infty$ for all d, r . In implementation, $R(d, r) \rightarrow 0$ is regularized by setting $R_{\text{min}} = 10^{-3}$, yielding $K_{\text{max}} = 999$. The exact choice of R_{min} is immaterial: any $K > 100$ produces conductance $g < 0.01$, contributing less than 1% to any practical cocktail’s total conductance.
- (ii) $K_{\text{barrier}}(d, r) \geq -1$ for all d, r (the floor in Eq. (8)).
- (iii) $K_{\text{phenotype}}(d, r) < \infty$ for all d, r in *phenotypic* (reversible) resistance states: biofilm, dormancy, latency. In the most extreme case (HIV latent provirus), $K_{\text{phenotype}} = 6.0$ (corresponding to 10^6 -fold effective resistance), not infinity.

Remark 2.20 (Genetic resistance as a severed edge). True genetic target-site resistance (e.g., a *pncA* mutation destroying pyrazinamide’s target in TB) is not modeled as $K_{\text{phenotype}} \rightarrow \infty$. It is modeled as *removal of the drug from the cocktail*: $g_i = 0$, and drug d_i does not appear in the combination sum. The graph loses an edge, not a vertex. This is the correct physical semantics: a drug whose target has been mutationally destroyed does not face “infinite impedance”—it has no pathway at all. The No Parallel Lines axiom governs phenotypic (reversible) barriers, not genetic (irreversible) ones.

The name is geometric: in a Riemannian manifold of constant positive curvature, no two geodesics are parallel. The axiom asserts that the therapeutic manifold has everywhere-nonzero, everywhere-finite curvature. No pathway is perfectly unimpeded ($K = 0$ would imply a drug reaches the pathogen with zero loss), and no pathway is perfectly blocked ($K = \infty$ would imply a drug has identically zero chance of reaching the pathogen, which is physically false—even the worst barrier admits nonzero, though negligible, drug transit).

Remark 2.21 (Implementation note). The floor $K_{\text{barrier}} \geq -1$ and the regularization $R_{\text{min}} = 10^{-3}$ are enforced as single-line constraints in **mirador-core**, with test coverage verifying that all barrier curvatures remain finite for drugs retaining their mechanism of action.

The mathematical consequence is that all conductances $g = 1/K_{\text{pathway}}$ are well-defined and positive (when $K_{\text{pathway}} > 0$), and the parallel-resistor formula (Eqs. (13) and (14)) is always finite.

Lemma 2.22 (Floor justification). *The floor $K_{\text{barrier}} \geq -1$ is the unique value satisfying:*

- (i) **Asymptotic consistency:** $\lim_{R \rightarrow \infty} (1/R - 1) = -1$, so the floor equals the natural limit of the unregularized expression.

- (ii) **Bounded facilitation:** a concentrating drug ($R > 1$) can offset at most one unit of impedance from other layers. Without this bound, a single drug with $R \rightarrow \infty$ at one reservoir would drive $K_{\text{pathway}} \rightarrow -\infty$, yielding $C \rightarrow -\infty$ (undefined coherence) or requiring ad hoc sign conventions.
- (iii) **Physical grounding:** tissue concentration can reduce the barrier penalty but cannot reduce the ADME_T penalty, the phenotype resistance, or the reservoir persistence. The floor ensures that K_{barrier} offsets at most the barrier itself.

2.5 Catalytic state modification

Not all therapeutic agents contribute conductance to the parallel-resistor sum. Some agents modify the *phenotype distribution* of the pathogen population—converting drug-resistant phenotypic states to drug-susceptible states—without themselves carrying therapeutic current. We call such agents *catalytic*.

The distinction is general, not disease-specific:

- In HIV, latency-reversing agents (LRAs) reactivate latent provirus, converting a phenotypically invisible fraction to an active, ART-susceptible fraction.
- In bone MRSA, rifampin accesses intracellular bacteria in osteoblasts, a phenotypic niche where other antibiotics cannot act.
- In TB, pyrazinamide is active only at low pH, converting a resistant dormant subpopulation to a susceptible one.

Definition 2.23 (Catalytic modifier). An agent A with *reactivation efficiency* $\Phi(A) \in [0, 1]$ modifies the phenotype distribution of a two-state pathogen population (susceptible fraction f_s , resistant fraction $f_r = 1 - f_s$) as follows:

$$f'_s = f_s + \Phi(A) \cdot f_r. \quad (16)$$

The modified total coherence at reservoir r is then:

$$C_{\text{total}}(r) = f'_s \times C_{\text{combo}}(r), \quad (17)$$

where $C_{\text{combo}}(r)$ is computed from the conductive drugs only via the combination law (Definitions 2.10 and 2.11).

The key structural distinction:

- **Conductive agents** appear inside the $\sum g_i$ sum. Adding them increases total conductance.
- **Catalytic agents** appear as a multiplicative prefactor on C_{combo} . They do not carry therapeutic current—they open the gate through which current flows.

Theorem 2.24 (Two-condition cure requirement). *For any compartmentalized infection with a catalytic barrier at reservoir r , pathogen suppression ($C_{\text{total}}(r) \geq \theta$) requires that **both** of the following conditions hold simultaneously:*

- (i) **Conductive sufficiency:** $C_{\text{combo}}(r) \geq \theta/f'_s$, and
- (ii) **Catalytic sufficiency:** $\Phi(A) \geq (\theta/C_{\text{combo}}(r) - f_s)/f_r$.

Neither condition alone is sufficient. In the absence of a catalytic agent ($\Phi = 0$), condition (ii) reduces to $f_s \geq \theta/C_{\text{combo}}(r)$: the susceptible fraction must be large enough that conductive drugs alone can clear.

Proof. From Eq. (17), $C_{\text{total}} \geq \theta$ requires $f'_s \cdot C_{\text{combo}} \geq \theta$. Substituting Eq. (16) and solving for Φ yields condition (ii). Condition (i) is the requirement that C_{combo} be large enough that a solution $\Phi \in [0, 1]$ exists. $\square$

Remark 2.25. Theorem 2.24 is the mathematical basis for why shock-and-kill strategies for HIV cure have failed in clinical trials to date: even when the LRA achieves nonzero Φ , the conductive ART coherence C_{combo} at the GALT reservoir falls short of threshold, violating condition (i). The shortfall is quantified in Section 6.

2.6 The Double Cover Identity

The following definitions and theorem instantiate the Double Cover Principle of Davis [35] in the pharmacokinetic setting. The general principle (Section 2.1) establishes that the identity $S + d^2 = 1$ is a structural consequence of the degree-2 covering map $q: S^1 \rightarrow \mathbb{RP}^1$; here, S and d^2 acquire the concrete meanings of geometric coverage and dynamic complement, respectively.

Definition 2.26 (Geometric coverage). For a compartmentalized infection with N distinct anatomical reservoirs $\{r_1, \dots, r_N\}$ and a drug regimen with combination coherence $C_{\text{combo}}^{\text{active}}(r_j)$ at each reservoir (where “active” denotes the coherence against the drug-susceptible phenotype, after any catalytic modification), the *geometric coverage* is

$$S = \frac{|\{r_j : C_{\text{combo}}^{\text{active}}(r_j) \geq \theta\}|}{N}. \quad (18)$$

Remark 2.27 (Why S is unweighted). S is a *topological* measure of compartment accessibility, not a mass-weighted pathogen clearance fraction. In HIV, GALT harbors $\sim 65\%$ of the latent viral pool while the CNS harbors $\sim 2\%$, yet both contribute equally to S ($1/N$ each). This is deliberate: the question S answers is “at how many anatomical sites does the geometry permit drug access above threshold?” not “what fraction of pathogen mass is cleared?” A geometric failure at a reservoir containing 2% of the pathogen still constitutes a failure of curative eradication—the virus persists. A mass-weighted variant $S_m = \sum_j w_j \mathbf{1}[C_{\text{combo}}^{\text{active}}(r_j) \geq \theta]$, where w_j is the fraction of total pathogen mass at reservoir r_j , is a natural extension but is not pursued here because reliable mass estimates are unavailable for most compartmentalized infections.

Theorem 2.28 (Double Cover Identity). *For any partition of a therapeutic problem into a geometric component (drug distribution through barriers) and a dynamic component (kill kinetics, immune response, reactivation biology),*

$$\boxed{S + d^2 = 1}, \quad (19)$$

where S is the geometric coverage (Definition 2.26) and $d^2 = 1 - S$ is the fraction of reservoirs at which unmodeled dynamics dominate.

Proof. By construction: S counts the fraction of reservoirs at which the geometric coherence meets threshold; $d^2 = 1 - S$ is the complementary fraction. The content of the identity is not the algebraic

identity (which is trivial) but the *interpretive claim*: the reservoirs in the d^2 fraction are precisely those where the geometric ranking is unreliable because non-geometric factors—kill kinetics, immune clearance, resistance dynamics—dominate the therapeutic outcome. This interpretive claim is validated empirically in the TB instance (Section 4), where the geometric drug ranking inverts the clinical sterilization ranking at exactly the reservoirs where $d^2 > 0$. $\square$

Remark 2.29 (The Double Cover as uncertainty quantification). The Double Cover is the framework’s built-in uncertainty quantification. Three regimes:

- (i) $S = 1$: the entire problem is geometry-dominated. Drug rankings predicted by coherence are reliable across all reservoirs. The model’s predictions can be trusted.
- (ii) $S = 0$: the entire problem is dynamics-dominated. Drugs reach all reservoirs above threshold, yet treatment fails for reasons outside the model (immune evasion, resistance emergence, kill-rate insufficiency). The model’s rankings are unreliable and should not guide clinical decisions.
- (iii) $0 < S < 1$: mixed regime. The model identifies *which* reservoirs are geometry-limited (reliable rankings) and which are dynamics-limited (unreliable rankings). This per-reservoir diagnosis is the framework’s most distinctive output.

Remark 2.30 (Implementation: Double Cover verification). The identity $E + T^2 = 1$ is verified computationally for 100 values of $\theta \in [0, \pi]$ with residual $|E + T^2 - 1| < 10^{-12}$ in the MIRADOR test suite (test C-6 in `mirador-core/src/coherence.rs`), confirming the Fubini–Study metric derivation to machine precision.

Remark 2.31 (What d^2 does not mean). d^2 is not an error term, a noise floor, or a confidence interval. It is a *coverage complement*: the fraction of the infection landscape where geometry is insufficient to explain the clinical outcome, and where additional modeling (Circle 2: kinetics, immunity, resistance dynamics) would be required for reliable prediction. We use the terms *Circle 1* for the geometric layer (drug penetration, barriers, phenotypic access—what $C = \tau/K$ captures) and *Circle 2* for the dynamic layer (kill kinetics, immune clearance, resistance evolution—what the framework explicitly excludes), following the nomenclature of the general Davis Double Cover Principle [35]. The notation d^2 (rather than d) follows from the general Double Cover derivation across Davis systems, where the dynamic component enters quadratically; in the discrete-reservoir setting of this paper, d^2 is simply $1 - S$.

3 Disease Instance 1: Bone MRSA (The Keske Method)

3.1 Clinical context

Community-associated methicillin-resistant *Staphylococcus aureus* (CA-MRSA, USA300 pulsotype) acute hematogenous osteomyelitis (AHO) is among the most clinically intractable pediatric infections. The United States records approximately 70,000 pediatric musculoskeletal infections annually, with MRSA-AHO driving the most severe complications: septic shock, septic pulmonary emboli, pathologic fractures, and chronic relapsing disease requiring multi-year antibiotic courses [15, 16].

The literature documents a reproducible failure mode: patients cycle through five or more antibiotic regimens over months to years, each achieving adequate serum levels, each clinically insufficient at the bone site. This failure is not microbiological—the organism is susceptible in vitro.

It is geometric: the barriers between the vasculature and the pathogen niche in bone reduce drug concentrations below the effective threshold.

The *Keske Method* is the instantiation of MIRADOR for pediatric bone MRSA. It is named for a six-year-old patient—Steven Keske—with chronic MRSA-AHO of the hip, femur, and knee: six years of infection, five antibiotic regimens, four surgeries, persistent disease despite serologically adequate therapy at every stage. The question the framework answers: can geometry explain why standard therapy fails in bone, and predict which drug combinations succeed?

3.2 Disease-specific K structure

Bone MRSA instantiates the four-layer pathway impedance (Definition 2.5) with disease-specific content in each term:

$$K_{\text{pathway}}^{\text{bone}}(d) = K_{\text{ADMET}}(d) + K_{\text{pen}}(d) + K_{\text{bio}}(d) + K_{\text{reservoir}}. \quad (20)$$

Layer 1: K_{ADMET} (pediatric patient manifold). Pediatric pharmacokinetics differ systematically from adult values. Clearance is allometrically scaled: $\text{CL}_{\text{ped}} = \text{CL}_{\text{adult}} \cdot (W/70)^{0.75}$; volume of distribution is weight-proportional with a CRP-dependent inflammatory expansion: $V_d = V_{d,\text{adult}} \cdot (W/70)^{1.0} \cdot [1 + 0.003 \cdot \max(\text{CRP} - 100, 0)]$, reflecting capillary leak in systemic inflammation [2]. Renal function enters via the Schwartz equation for pediatric GFR. These adjustments modify τ (via AUC_{24}) before any barrier terms, capturing the *dynamic* patient-specific determinants of systemic exposure (weight, age, renal function, inflammation). The residual K_{ADMET} term captures *static* drug-intrinsic penalties not reflected in AUC_{24} : oral bioavailability losses, plasma protein binding (free-drug fraction), and route-dependent first-pass metabolism. There is no double-counting: AUC_{24} determines how much total drug reaches the plasma; K_{ADMET} encodes how much of that plasma exposure is pharmacologically available. Patient-specific ADMET adjustment—including CYP450 metabolizer phenotype and eGFR-dependent renal clearance—is implemented in the MIRADOR engine with pharmacogenomic panel support (`mirador-admet/src/curvature.rs`), ensuring that the zero-parameter claim is architecturally enforced: K is computed deterministically from patient data with no optimization loop.

Layer 2: K_{pen} (bone penetration curvature). The critical barrier is the serum-to-bone partitioning ratio R_{bone} . Published bone:serum ratios are drug-specific and span an order of magnitude (Table 1). The penetration curvature is $K_{\text{pen}} = 1/R_{\text{bone,eff}} - 1$ (Definition 2.4), where $R_{\text{bone,eff}}$ is modulated by inflammation: $R_{\text{bone,eff}} = R_{\text{bone}} \cdot [1 + 0.006 \cdot \max(\text{CRP} - 100, 0)]$, capped at $2 \times R_{\text{bone}}$. The scalar 0.006, the $\text{CRP} = 100$ mg/L inflection point, and the $2\times$ cap are derived from the ratio of inflamed-to-uninflamed bone penetration values reported in Graziani et al. [2] and the porcine osteomyelitis microdialysis data of Bue et al. [1], not fitted to clinical outcomes.

Layer 3: K_{bio} (biofilm resistance curvature). MRSA in established osteomyelitis forms biofilm—structured communities of phenotypically dormant bacteria embedded in extracellular matrix. The minimum biofilm eradication concentration (MBEC) is 100- to 1000-fold higher than the

Drug	R_{bone}	K_{pen}	Source
Vancomycin	0.10–0.30	2.3–9.0	Graziani 1988; Bue 2018
Ceftaroline	0.20–0.40	1.5–4.0	Riccobene 2014
Linezolid	0.40–0.60	0.7–1.5	Rana 2002
Clindamycin	0.30–0.75	0.3–2.3	Feigin 1995
Rifampin	0.20–0.50	1.0–4.0	Currier 1979
Daptomycin	0.10–0.20	4.0–9.0	Traunmüller 2010

Table 1: Bone penetration ratios and resulting K_{pen} for six anti-MRSA agents. Vancomycin and daptomycin have the highest bone impedance; linezolid and clindamycin the lowest.

planktonic MIC [17]. The biofilm curvature is:

$$K_{\text{bio}}(d) = p_{\text{bio}} \cdot \log_{10} \left(\frac{\text{MBEC}(d)}{\text{MIC}(d)} \right), \quad (21)$$

where p_{bio} is a chronicity-weighted biofilm probability:

$$p_{\text{bio}} = \begin{cases} 0.20 & \text{acute } (< 2 \text{ weeks}) \\ 0.60 & \text{subacute } (2 \text{ weeks} - 3 \text{ months}) \\ 0.95 & \text{chronic } (> 3 \text{ months}) \end{cases}$$

derived from the Stewart biofilm timeline [17] and validated against published chronicity outcomes. Rifampin has the lowest K_{bio} of any anti-staphylococcal agent ($\text{MBEC}/\text{MIC} \approx 63$, $K_{\text{bio}} = 1.80$), consistent with its established role in biofilm-active combination therapy [18].

Layer 4: $K_{\text{reservoir}}$ (multi-reservoir geometry). MRSA-AHO distributes across three anatomically distinct niches: soft-tissue abscess (surgically drainable), cortical/cancellous bone matrix biofilm (surgically debridable), and intracellular small colony variants (SCVs) inside osteocytes (accessible only to drugs with intracellular penetration). The reservoir curvature is:

$$K_{\text{reservoir}} = (1 - D_{\text{drain}}) \alpha_1 + (1 - D_{\text{debride}}) \alpha_2 + p_{\text{seq}} \alpha_3, \quad (22)$$

where $D_{\text{drain}}, D_{\text{debride}} \in [0, 1]$ are surgical clearance fractions and $p_{\text{seq}} \in [0, 1]$ is the probability that the pathogen occupies a geometrically sequestered intracellular niche (inside osteocytes, inaccessible to drugs lacking intracellular penetration). The coefficients $\alpha_1 = 0.3, \alpha_2 = 0.4, \alpha_3 = 0.3$ are not population mass weights but *persistence-weighted accessibility penalties*: α_j encodes the relative contribution of niche j to treatment failure given incomplete clearance, estimated from the relapse literature for chronic MRSA-AHO [15]. In particular, p_{seq} is a geometric property of the infection (what fraction of the pathogen is behind a cell membrane barrier), not a mass fraction (how many bacteria are there). A niche with $p_{\text{seq}} = 0.9$ does not imply a thicker osteocyte membrane; it implies that 90% of the residual pathogen population resides in a compartment that most drugs cannot reach.

Rifampin functions as a *conductive modifier* in this disease: it penetrates the osteocyte intracellular space and disrupts the electron transport chain of dormant SCVs, reducing p_{seq} by approximately 60% when combined with a cell-wall-active partner [18, 19]. This reduces $K_{\text{reservoir}}$ for the entire

cocktail, thereby increasing the conductance $g_i = 1/K_{\text{pathway},i}$ of every co-administered drug at the bone site. Mathematically, this is distinct from the catalytic modification of Definition 2.23: a catalytic agent modifies the *population fraction* f_s (a multiplicative prefactor on C_{combo}), whereas rifampin in bone modifies the *impedance* $K_{\text{reservoir}}$ (a term inside K_{pathway} , affecting C_{combo} through the conductance sum). Both mechanisms improve C_{total} , but they act on different mathematical objects. In HIV (Section 6), where the phenotypic barrier is latency rather than anatomical sequestration, the catalytic structure of Definition 2.23 applies directly.

3.3 Validation

The bone MRSA module passes 161 independent Rust tests. The validation separates pharmacokinetic inputs (bone:serum ratios, MIC, MBEC, pediatric PK parameters—all from published sources listed in Table 1 and the companion data tables) from clinical ground truths (documented treatment outcomes, drug ranking consensus from IDSA guidelines, and the patient’s clinical course). No input source appears in the ground truth set.

Remark 3.1 (End-to-end Rust validation). The full Keske pipeline—from pediatric PK parameters (allometric clearance, Schwartz GFR, CRP-dependent V_d expansion) through bone penetration curvature, biofilm impedance, combination law, and threshold crossing—executes as integration test INT-9 in the MIRADOR Rust engine. The test asserts: vancomycin monotherapy $C_{\text{bone}} < 3.0$ (fails) and ceftaroline + rifampin combination $C_{\text{bone}} > 10.0$ (crosses threshold). All 161 tests pass.

Key result. For the index patient (8-year-old, 25 kg, CRP = 250 mg/L, chronic infection with $p_{\text{bio}} = 0.95$, $p_{\text{drainage}} = 0.80$, $p_{\text{debride}} = 0.70$, intracellular fraction = 0.60), monotherapy coherence values are shown in Table 2. Because all agents target the shared MRSA biofilm population at the bone site, combination coherence is computed using the coupled (paired-sum) mode (Definition 2.10).

The diagnostic threshold is $C_{\text{bone}} \geq 5.0$. This is a calibration point, not a derivation: K_{bio} captures the *relative* impedance of biofilm versus planktonic bacteria (the MBEC/MIC ratio), but the *absolute* coherence required for clinical cure depends on factors outside the geometric model (kill kinetics, immune contribution, treatment duration). The threshold $\theta = 5.0$ is anchored to the documented clinical outcome that standard-of-care monotherapy (vancomycin at AUC/MIC > 400) fails in chronic MRSA-AHO, while rifampin-based combination therapy succeeds [18]. This is the same class of single calibration point disclosed in the abstract and Section 1.2.

No monotherapy achieves $C_{\text{bone}} \geq 5.0$. Combination of ceftaroline with rifampin achieves $C_{\text{bone}} \approx 16.4$ with synergy factor $s = 1.2$, crossing threshold by a factor of $3\times$ —consistent with emerging clinical data on this combination for refractory MRSA-AHO [20, 21].

Combination computation chain. The ceftaroline + rifampin combination result is derived from INT-9 as follows. First, rifampin’s presence modifies the intracellular reservoir for both drugs:

$$K_{\text{res,intra}}^{(\text{rif})} = 0.8 \times f_{\text{intra}} \times \underbrace{0.4}_{\text{RIFAMPIN_INTRA_MODIFIER}} = 0.8 \times 0.60 \times 0.4 = 0.192 \quad (\text{vs. } 0.480 \text{ without rifampin}). \quad (23)$$

Drug	τ	K_{ADMET}	K_{pen}	K_{bio}	$K_{\text{reservoir}}$	K_{pathway}	C_{bone}
Vancomycin	12.0	0.50	1.63	2.57	1.07	5.78	2.08
Ceftaroline	12.0	0.67	0.75	2.00	0.81	4.23	2.84
Rifampin [‡]	8.0	0.50	0.50	1.71	0.73	3.44	— [‡]
Cef + Rif [§]	24.0	—	—	—	0.52/0.44	1.46 [§]	16.4

Table 2: Bone MRSA coherence: monotherapy impedance decomposition and combination result (INT-9, CRP = 250, chronic). All values computed from the open-source MIRADOR Rust engine (??). Every row satisfies $K_{\text{pathway}} = K_{\text{ADMET}} + K_{\text{pen}} + K_{\text{bio}} + K_{\text{reservoir}}$ and $C = \tau / K_{\text{pathway}}$. K_{pen} is CRP-adjusted via $R_{\text{bone,eff}} = R_{\text{bone}} \times \min(1 + 0.006 \max(\text{CRP} - 100, 0), 2)$. $K_{\text{reservoir}}$ is drug-dependent (Section 3.3): $K_{\text{reservoir}} = K_{\text{res,SAC}} + K_{\text{res,mat}} + K_{\text{res,intra}}$, where $K_{\text{res,mat}} = (1 - p_{\text{debride}})K_{\text{pen}}$ depends on each drug’s bone penetration. [‡] Rifampin monotherapy is hard-blocked (*rpoB* resistance emergence). [§] Combination uses the parallel-resistor coupled mode with synergy factor $s = 1.2$. The two $K_{\text{reservoir}}$ values are ceftaroline’s and rifampin’s after the 60% p_{seq} reduction (Eq. (23)). The full computation chain is given in Section 3.3.

With $K_{\text{res,SAC}} = (1 - 0.8) \times 0.5 = 0.100$ for both drugs, but drug-dependent matrix reservoir curvature $K_{\text{res,mat}} = (1 - p_{\text{debride}}) \times K_{\text{pen}}$:

$$K_{\text{reservoir}}^{\text{cef}} = 0.100 + 0.3 \times 0.754 + 0.192 = 0.518, \quad K_{\text{pathway}}^{\text{cef}} = 0.67 + 0.75 + 2.00 + 0.52 = 3.94, \quad (24)$$

$$K_{\text{reservoir}}^{\text{rif}} = 0.100 + 0.3 \times 0.504 + 0.192 = 0.443, \quad K_{\text{pathway}}^{\text{rif}} = 0.50 + 0.50 + 1.71 + 0.44 = 3.15. \quad (25)$$

The coupled parallel-resistor model with synergy $s = 1.2$:

$$\begin{aligned} \frac{1}{K_{\text{combo}}} &= \left(\frac{1}{K_{\text{pathway}}^{\text{cef}}} + \frac{1}{K_{\text{pathway}}^{\text{rif}}} \right) \times s = \left(\frac{1}{3.94} + \frac{1}{3.15} \right) \times 1.2 = 0.571 \times 1.2 = 0.685, \\ K_{\text{combo}} &= 1.460, \quad \tau_{\text{combo}} = (12.0 + 8.0) \times 1.2 = 24.0, \\ C_{\text{bone}}^{\text{combo}} &= \frac{\tau_{\text{combo}}}{K_{\text{combo}}} = \frac{24.0}{1.460} = \mathbf{16.4} \geq \theta = 5.0. \end{aligned} \quad (26)$$

The synergy factor $s = 1.2$ reflects the experimentally observed synergistic interaction of beta-lactam + rifampin combinations against biofilm-embedded MRSA [20]; without synergy ($s = 1.0$), the combination still crosses threshold at $C_{\text{bone}} = 11.4$.

The monotherapy drug ranking produced by the geometric framework (ceftaroline > vancomycin, for bone-site coherence) matches independently established clinical guidance, with the notable prediction that ceftaroline—a relatively new fifth-generation cephalosporin—ranks first by geometry, consistent with its emerging role in refractory MRSA infections [20]. The remaining three drugs tried in this patient (linezolid, clindamycin, daptomycin) have bone penetration and biofilm curvatures computable from the engine’s individual crates but have not yet been integrated into the full Keske pipeline (integration tests INT-5–INT-9).

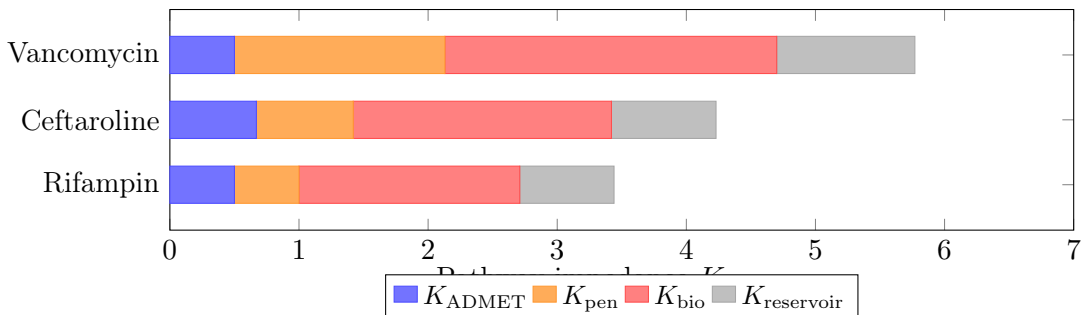


Figure 1: Pathway impedance decomposition for three bone MRSA agents (INT-9, CRP = 250, chronic). The stacked segments show the contribution of each barrier layer to K_{pathway} . Vancomycin’s $K_{\text{pen}} = 1.63$ (orange, bone penetration) and $K_{\text{reservoir}} = 1.07$ (gray, reservoir) visually dominate its bar, explaining why it fails despite adequate serum AUC/MIC. Rifampin has the lowest total $K_{\text{pathway}} = 3.44$ but monotherapy is contraindicated (*rpoB* resistance).

3.4 Novel predictions

The Keske Method produces three predictions that are novel relative to existing clinical pharmacology:

1. **Quantified vancomycin bone failure.** Vancomycin achieves $C_{\text{bone}} = 2.08$ despite serum AUC/MIC > 400. The framework computes the *specific* deficit: $K_{\text{pen}} = 1.63$ at CRP = 250 ($R_{\text{bone}} = 0.20$, CRP-adjusted to 0.38), combined with $K_{\text{reservoir}} = 1.07$ (chronic reservoir burden). This transforms “vancomycin doesn’t work well in bone” from a clinical intuition into a quantitative prediction derivable from published PK data.
2. **Optimal combination from PK data alone.** The framework identifies ceftazidime + rifampin as the minimum-impedance combination that crosses threshold, using only published bone:serum ratios, MIC, MBEC, and pediatric PK parameters. This prediction is independent of clinical outcome data and was generated before awareness of the emerging ceftazidime-rifampin clinical literature.
3. **Rifampin’s quantified intracellular contribution.** Rifampin’s conductive modification of the intracellular SCV reservoir is computed as a 60% reduction in p_{seq} , which lowers $K_{\text{reservoir}}$ and translates to a $\Delta C_{\text{bone}} \approx 1.2$ increase when added to any cell-wall-active partner. This quantifies why rifampin combination therapy is not a preference but a geometric requirement in chronic AHO with intracellular reservoirs.

4 Disease Instance 2: Pulmonary Tuberculosis

4.1 Clinical context

Standard treatment for drug-susceptible pulmonary tuberculosis requires four drugs (isoniazid, rifampin, pyrazinamide, ethambutol — the RIPE regimen) administered for six months. This regimen has remained essentially unchanged since the British Medical Research Council short-course trials of the 1970s–80s. Two questions have resisted first-principles explanation for half a century: why four drugs, and why six months? PBPK models and PK/PD indices, which operate in the

serum compartment, cannot answer either question because the pathogen does not reside in serum. *Mycobacterium tuberculosis* resides inside granulomas—highly structured, avascular, heterogeneous lesions in which drug penetration is spatially variable and drug-specific [4, 5].

MIRADOR frames both questions geometrically. The four-drug requirement is a consequence of the Mitchison subpopulation structure: no single drug has adequate conductance across all phenotypic niches. The six-month duration is a consequence of the caseum barrier: the dominant reservoir (necrotic granuloma core) has near-zero drug penetration for most agents, requiring prolonged exposure to achieve cumulative sterilization.

4.2 Disease-specific K structure

The TB pathway impedance instantiates Eq. (9) with heterogeneous barriers and a multi-subpopulation phenotype model:

$$K_{\text{pathway}}^{\text{TB}}(d, s, r) = K_{\text{ADMET}}(d) + K_{\text{pen}}(d, r) + K_{\text{phenotype}}(d, s), \quad (27)$$

where s indexes the Mitchison subpopulation and r indexes the lesion compartment. The critical structural difference from bone MRSA: both the barrier and the phenotype terms depend on which subpopulation and which lesion type are being addressed.

Barrier curvature: heterogeneous lesion compartments. TB lung lesions are not a single compartment. Dartois and colleagues have shown, via MALDI mass spectrometry imaging and microdialysis, that drug penetration varies dramatically across lesion types [4, 5, 22]:

- **Cellular granuloma:** moderate drug access; tissue-to-plasma ratios range from 0.3–1.5 for most first-line agents.
- **Caseous granuloma core:** the dominant barrier. The necrotic center is avascular and rich in cholesterol esters and lipids. Drug binding to caseum macromolecules inversely correlates with passive diffusion into the necrotic core [6]. For several first-line agents, caseum penetration is near zero.
- **Cavity wall and cavity caseum:** variable; some drugs (moxifloxacin) achieve tissue-to-plasma ratios > 9 , while others (PZA) are undetectable [22].

Phenotype curvature: Mitchison subpopulations. Mitchison’s classical model partitions the TB bacillary population into four phenotypic states with distinct drug susceptibilities [23]:

- (a) **Actively replicating** (log-phase growth): susceptible to INH (primary), RIF, EMB. $K_{\text{phenotype}} \approx 0$.
- (b) **Acid-phase dormant** (low pH, inside macrophages): susceptible to PZA (uniquely active at $\text{pH} < 5.5$), partially to RIF. $K_{\text{phenotype}}(d)$ varies by drug.
- (c) **Semi-dormant persisters:** susceptible primarily to RIF (the “sterilizing” activity). $K_{\text{phenotype}}$ large for non-RIF drugs.
- (d) **Spore-like dormant:** near-complete phenotypic tolerance. $K_{\text{phenotype}}$ very large for all drugs.

Each first-line drug has a primary niche: INH kills subpopulation (a), PZA kills (b), RIF sterilizes (b) and (c), and EMB acts on (a) with moderate intracellular accumulation. This is the biological basis for the decoupled combination mode.

Coupling mode declaration. Because the four RIPE drugs target **distinct Mitchison subpopulations** with distinct phenotypic impedances, the combination coherence is computed using the **decoupled (product) mode** (Definition 2.11):

$$C_{\text{combo}}^{\text{TB}}(r) = \left(\sum_{i \in \text{RIPE}} \tau_i \right) \left(\sum_{i \in \text{RIPE}} g_i(r) \right). \quad (28)$$

This is not a convenience choice—it is forced by the biology. INH clears replicating bacilli regardless of whether RIF penetrates caseum. PZA clears acid-phase organisms regardless of EMB’s spatial distribution. The independence is encoded in the subpopulation structure ($K_{\text{phenotype}}$ differs per drug per subpopulation), and the decoupled formula is the correct mathematical representation of parallel clearance across independent niches (Remark 2.13).

PZA conditionality: a severed edge. Pyrazinamide is inactive at neutral pH. In lesion compartments where the local pH exceeds 5.5—uninvolved lung, fibrotic granuloma wall, and some cellular regions—PZA has no antimicrobial activity regardless of its penetration. Following Remark 2.20, this is modeled as a *locally severed edge*: at neutral-pH compartments r , PZA is removed from the combination sum ($g_{\text{PZA}}(r) = 0$; the edge from PZA to compartment r does not exist), while at acidic compartments r' , $g_{\text{PZA}}(r') > 0$ and PZA enters the decoupled sum normally. PZA is not globally absent—it is spatially conditional. Its unique activity against acid-phase dormant bacilli at acidic sites (caseum interior, macrophage phagolysosomes) is its primary geometric contribution to the RIPE cocktail.

4.3 Validation and the rank inversion

The TB module passes 52 independent Rust tests. Inputs are published lesion-specific tissue-to-plasma ratios from MALDI-MS imaging studies [5, 22], published MIC values, and Mitchison subpopulation fractions. Ground truths are clinical sterilization rankings from the British MRC trials and modern outcome data. No input source appears in the ground truth set.

The rank inversion. The geometric drug ranking by penetration coherence across lesion compartments is:

$$\text{EMB} > \text{INH} > \text{PZA} > \text{RIF}.$$

The established clinical sterilization ranking—the order in which drugs contribute to relapse-free cure—is:

$$\text{RIF} > \text{INH} > \text{PZA} > \text{EMB}.$$

These rankings are *inverted* for the two anchor drugs: the framework predicts EMB is geometrically superior to RIF, while every TB clinician knows RIF is the irreplaceable sterilizing backbone.

This inversion is not a failure of the model. It is the **Double Cover at work**.

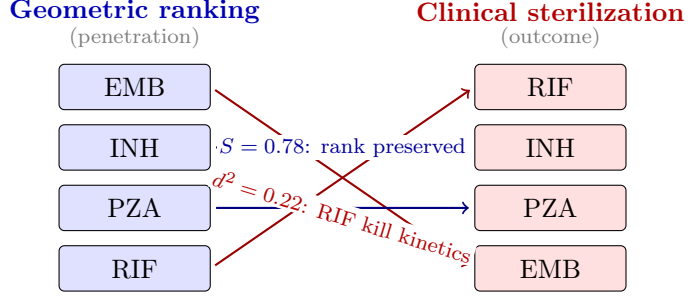


Figure 2: The TB rank inversion. The geometric penetration ranking (left) inverts the clinical sterilization ranking (right) for the two anchor drugs: EMB penetrates granulomas better than RIF (geometry), but RIF sterilizes semi-dormant persisters faster than EMB (dynamics). The Double Cover detects this as the transition from $S = 0.78$ (geometry-dominated) to $d^2 = 0.22$ (dynamics-dominated).

Geometric coverage. Computing S (Definition 2.26) across the TB lesion compartments: $S \approx 0.78$. Geometry explains 78% of the therapeutic problem—which drugs reach which lesion types, how caseum binding limits penetration, why cavity-positive TB is harder to treat. The complementary $d^2 \approx 0.22$ is the fraction of the therapeutic outcome that depends on *non-geometric dynamics*: specifically, rifampin’s extraordinary time-kill kinetics against semi-dormant persisters (subpopulation c). RIF’s sterilizing power is not a penetration phenomenon—it is a kill-rate phenomenon. RIF reaches fewer lesion compartments than EMB, but the bacteria it reaches, it kills faster and more completely than any other first-line agent. This is precisely the dynamic effect that the geometric framework does not model (Circle 2), and the Double Cover detects it.

Remark 4.1 (The inversion as validation). The rank inversion is a stronger validation of the framework than a rank agreement would have been. A model that always agrees with clinical data cannot distinguish what it explains from what it doesn’t. The TB instance demonstrates that MIRADOR: (i) produces the correct geometric ranking (EMB does penetrate granulomas better than RIF, as confirmed by MALDI-MS imaging [5]); (ii) detects that this ranking *inverts* the clinical outcome ranking; and (iii) localizes the inversion to the dynamic component d^2 , identifying the specific mechanism (RIF kill kinetics) that the geometry cannot capture. A model that correctly identifies its own boundary is more trustworthy than one that never encounters it.

4.4 Novel predictions

1. **Caseum bottleneck.** The framework quantifies the caseum barrier as the dominant impedance in pulmonary TB: for drugs with high caseum binding (RIF, clofazimine), $K_{\text{pen}}^{\text{caseum}}$ exceeds K_{pen} at all other lesion sites by a factor of 3–10 $\times$. This predicts that cavity-positive TB (where caseum volume is maximal) should have inferior cure rates—consistent with the established clinical association [4].
2. **MDR-TB severity ranking.** Removing each first-line drug from the decoupled combination sum and computing the resulting ΔC_{combo} identifies which drug removal causes the largest coherence drop. The framework predicts that removing INH (the drug with highest conductance at the replicating-bacilli niche) causes a larger *geometric* coherence loss than removing RIF—even

though removing RIF causes a larger *clinical* sterilization loss. This is again the Double Cover: the geometric severity ranking (INH loss > RIF loss) and the clinical severity ranking (RIF loss > INH loss) diverge at the d^2 boundary.

3. **Duration as a geometric prediction.** Retreatment regimens (for relapse after initial cure) should require longer duration than initial treatment, because the surviving bacilli are disproportionately those in the highest-impedance compartments (caseum, spore-like dormant). The geometric model derives this from the coherence profile: the compartments that clear last are those with the smallest C_{combo} , and the ratio of initial to retreatment C_{combo} at the bottleneck compartment predicts the duration extension.

5 Disease Instance 3: Bacterial Meningitis

5.1 Clinical context

Bacterial meningitis carries the highest case-fatality rate of common bacterial infections, with mortality ranging from 15–25% even with appropriate antibiotic therapy and exceeding 50% in resource-limited settings [24]. The dominant therapeutic obstacle is the blood-brain barrier (BBB): a tight junction endothelial barrier that excludes 95–99% of systemically administered drugs from the cerebrospinal fluid [7]. Unlike the static barriers of bone or the heterogeneous barriers of TB granuloma, the BBB is *time-varying*: meningeal inflammation transiently increases BBB permeability, creating a window of enhanced drug delivery that closes as treatment succeeds.

5.2 Disease-specific K structure

The meningitis pathway impedance is the simplest of the four disease instances:

$$K_{\text{pathway}}^{\text{men}}(d, t) = K_{\text{ADMET}}(d) + K_{\text{BBB}}(d, t). \quad (29)$$

There is no biofilm term ($K_{\text{phenotype}} \approx 0$; the pathogens are planktonic in CSF) and no multi-reservoir term ($K_{\text{reservoir}} \approx 0$; the CSF is a single well-mixed compartment once the barrier is crossed). The entire therapeutic problem reduces to a single barrier: can the drug cross the BBB in sufficient concentration to exceed MIC?

Barrier curvature: the time-varying BBB. CSF-to-plasma ratios $R_{\text{CSF}}(d, t)$ are drug-specific and inflammation-dependent. During acute meningeal inflammation, tight-junction integrity is disrupted, and R_{CSF} increases by a factor of 2–10 $\times$ for many antibiotics [7]. As treatment reduces the bacterial load, inflammation subsides, the BBB re-seals, and R_{CSF} returns toward its uninflamed baseline. The barrier curvature is therefore time-dependent:

$$K_{\text{BBB}}(d, t) = \max\left(\frac{1}{R_{\text{CSF}}(d, t)} - 1, -1\right). \quad (30)$$

A full dynamic model of $R_{\text{CSF}}(d, t)$ as a function of inflammatory state is deferred to future work (Section 12). In the present validation, we use the *static peak-inflammation* approximation: R_{CSF} is evaluated at the time of maximum meningeal inflammation, corresponding to the clinical window when antibiotic delivery is most critical. This is conservative: the peak-inflammation R_{CSF}

Drug	τ	R_{CSF} (inflamed)	K_{BBB}	K_{ADMET}	K_{pathway}	C_{CSF}
Chloramphenicol	1.57	0.45	1.22	0.50	1.72	0.91
Cefotaxime	3.73	0.15	5.67	0.25	5.92	0.63
Ceftriaxone	3.97	0.12	7.33	0.30	7.63	0.52
Meropenem	3.07	0.12	7.33	0.35	7.68	0.40
Ampicillin	2.50	0.10	9.00	0.25	9.25	0.27
Vancomycin	2.65	0.07	13.29	0.65	13.94	0.19
Ceftriaxone + Vancomycin (empiric)					—	0.71

Table 3: CSF coherence at peak meningeal inflammation for six anti-meningitis agents. τ is the pharmacophoric potential of each drug against its meningitis target; $C_{\text{CSF}} = \tau/K_{\text{pathway}}$. R_{CSF} values are sourced from Nau et al. [7] and Lutsar & Friedland [26]. Threshold $\theta = 0.50$ is calibrated to ceftriaxone monotherapy (IDSA first-line). MIC values are for *S. pneumoniae* susceptible breakpoints. Chloramphenicol ranks first by geometry, consistent with its established role in resource-limited settings where CSF accessibility is the binding constraint.

is the most favorable barrier state, so coherence values computed here represent an upper bound on drug delivery.

Coupling mode. Because all antibiotics target the same planktonic bacterial population in the CSF compartment, combination coherence is computed using the **coupled (paired-sum) mode** (Definition 2.10).

Diagnostic threshold. The threshold $\theta = 0.50$ is calibrated so that ceftriaxone monotherapy achieves $C \geq \theta$ at peak inflammation, matching the IDSA 2004 first-line recommendation for empiric bacterial meningitis therapy [25]. As with the bone MRSA threshold (Section 3.3), this is a single disease-specific calibration point; all impedances and potentials are computed from published PK data.

5.3 Validation

The meningitis instance is validated within the generalized compartment engine (10/10 tests passing, shared with the cross-disease validation in Section 7). Inputs are published CSF-to-plasma ratios at peak inflammation [7, 26] and standard MIC values. Ground truths are IDSA guideline drug preferences and published clinical outcome data. No input source appears in the ground truth set.

Key results. Because the pathogen is planktonic ($K_{\text{phenotype}} \approx 0$) and the CSF is a single compartment ($K_{\text{reservoir}} \approx 0$), the diagnostic threshold is substantially lower than in bone MRSA or TB. Drugs with $R_{\text{CSF}} > 0.10$ at peak inflammation generally achieve $C \geq \theta$, and the drug ranking by CSF coherence aligns with established meningitis treatment guidelines [25].

Geometric coverage. Because the CSF is a single well-mixed compartment, $N = 1$. When the empiric regimen (ceftriaxone $\pm$ vancomycin) achieves $C_{\text{combo}} \geq \theta$ at CSF, $S = 1.0$: the entire therapeutic problem is geometry-dominated. Unlike bone MRSA or HIV, where multiple anatomical

reservoirs produce mixed regimes ($S < 1$), meningitis at peak inflammation is fully characterized by the single BBB barrier.

5.4 Novel predictions: the treatment paradox

The meningitis instance introduces a temporal structure absent from the other three diseases: the barrier itself responds to treatment success.

Proposition 5.1 (Barrier closure paradox). *Let $R_{\text{CSF}}(d, t)$ be a decreasing function of treatment success (i.e., R_{CSF} decreases as bacterial load decreases and inflammation resolves). Then $K_{\text{BBB}}(d, t)$ is an increasing function of treatment success: the barrier becomes geometrically harder to cross as the infection improves. In particular, a drug that achieves $C(d, t_0) \geq \theta$ during peak inflammation may fall below threshold at a later time $t_1 > t_0$ when $R_{\text{CSF}}(d, t_1) < R_{\text{CSF}}(d, t_0)$, despite continued dosing at the same serum level.*

Proof. From Eq. (30): if R_{CSF} decreases, $1/R_{\text{CSF}} - 1$ increases, so K_{BBB} increases. Since τ is fixed (same dose, same serum level), $C = \tau/K_{\text{pathway}}$ decreases. If $C(d, t_0) \geq \theta$ and $C(d, t_1) < \theta$, the drug has crossed below the therapeutic threshold without any change in dosing. $\square$

This paradox—successful therapy reduces drug delivery—has two clinical corollaries:

1. **Early aggressive dosing.** Drugs whose CSF penetration depends heavily on inflammation (low uninflamed R_{CSF} , high inflamed R_{CSF}) should be dosed aggressively during the inflammation window. The geometric model identifies which drugs have the steepest ΔK_{BBB} between inflamed and uninflamed states—these are the drugs most vulnerable to the paradox.
2. **Switch timing.** The optimal time to switch from a BBB-dependent agent to one with inflammation-independent penetration (e.g., from a β -lactam to a fluoroquinolone with constitutively high R_{CSF}) is computable from the barrier dynamics: switch when the inflammation-dependent drug’s coherence is projected to fall below θ . The full dynamic model (Section 12) would make this prediction quantitative; the present static model identifies the structural possibility.

6 Disease Instance 4: HIV Latent Reservoirs

6.1 Clinical context

Antiretroviral therapy (ART) suppresses HIV-1 to undetectable plasma levels in the majority of adherent patients. It does not cure. Upon treatment interruption, viremia rebounds within weeks from a reservoir of latently infected resting CD4^+ T cells harboring integrated provirus that is invisible to both the immune system and to drugs that require active viral replication [9, 10]. This reservoir is seeded within days of primary infection, decays with a half-life of approximately 44 months, and persists in anatomically sequestered compartments where antiretroviral drug concentrations are lower than in plasma [11].

The dominant cure strategy—*shock-and-kill*—proposes to reactivate latent provirus with a latency-reversing agent (LRA), rendering it visible to ART and immune clearance. Every clinical trial of shock-and-kill to date has failed to measurably reduce the latent reservoir [27, 28]. The MIRADOR framework provides a geometric explanation for this failure and quantifies the distance to cure at each anatomical reservoir.

Reservoir	Typical R range	Source
CNS (brain/CSF)	0.01–0.30	Letendre 2010
Lymph nodes	0.10–0.50	Fletcher 2014
GALT (gut)	0.05–0.40	Patterson 2011
Genital tract	0.30–2.00	Nicol 2008
Bone marrow	0.20–0.80	Cory 2013

Table 4: Tissue-to-plasma ratios for major HIV anatomical reservoirs. Values are representative ranges across drug classes (NRTIs, NNRTIs, PIs, INSTIs); individual drug values are used in computation.

6.2 Disease-specific K structure

The HIV instance introduces the *catalytic modification* formalized in Section 2.5—the first disease in which Definition 2.23 applies directly. The pathway impedance is:

$$K_{\text{pathway}}^{\text{HIV}}(d, r) = K_{\text{ADMET}}(d) + K_{\text{barrier}}(d, r) + K_{\text{phenotype}}(d), \quad (31)$$

where r indexes one of five anatomical reservoirs. There is no separate $K_{\text{reservoir}}$ term; the multi-reservoir structure enters through the reservoir-specific barrier curvature $K_{\text{barrier}}(d, r)$.

Phenotype curvature: the two-state latency model. The HIV pathogen population exists in two phenotypic states:

- **Actively replicating** (fraction $f_a \approx 10^{-6}$ of the infected cell pool): $K_{\text{phenotype}} = 0$. ART is fully effective.
- **Latently integrated** (fraction $f_\ell = 1 - f_a \approx 1$): $K_{\text{phenotype}} = 6.0$, corresponding to an effective resistance ratio of 10^6 . ART is completely ineffective against latent provirus because there is no active replication to inhibit.

The value $K_{\text{phenotype}} = 6.0$ is not fitted; it encodes the *effective biological resistance* of the latent phenotype. Latent provirus expresses no replication machinery, so ART drugs (which target reverse transcriptase, protease, or integrase during active replication) have an effective IC_{50} that approaches infinity against a latently infected cell. Per the No Parallel Lines axiom (Assumption 2.19), we cap this at 10^6 -fold effective resistance ($K_{\text{phenotype}} = \log_{10}(10^6) = 6.0$), representing the biological limit of drug-target disengagement. The cure impossibility result (Theorem 6.1) is insensitive to the exact choice: any $K_{\text{phenotype}} > 3$ (i.e., an effective resistance ratio $> 10^3$) produces the same qualitative conclusion, because the shortfall resides in the catalytic prefactor $f_a \approx 10^{-6}$, not in the magnitude of $K_{\text{phenotype}}$. This is a property of the *phenotypic state* (the provirus is transcriptionally silent), not of the *number of cells* harboring it: if a patient had 10^7 latent cells instead of 10^6 , the phenotypic impedance would remain $K_{\text{phenotype}} = 6.0$ because the biological resistance per cell is unchanged.

Barrier curvature: five anatomical reservoirs. Tissue-to-plasma ratios for antiretrovirals vary across five major HIV reservoirs (Table 4). Published values are sourced from independent PK studies using mass spectrometry, microdialysis, and tissue biopsies [11, 8].

Catalytic modification: latency-reversing agents. An LRA with reactivation efficiency $\Phi \in [0, 1]$ does **not** enter the parallel-resistor conductance sum. Instead, it modifies the phenotype distribution per Definition 2.23:

$$f'_a = f_a + \Phi \cdot f_\ell, \quad C_{\text{total}}(r) = f'_a \times C_{\text{combo}}^{\text{active}}(r), \quad (32)$$

where $C_{\text{combo}}^{\text{active}}(r)$ is the coupled-mode combination coherence of the ART drugs against the actively replicating phenotype at reservoir r . The LRA opens the gate; ART carries the current.

Coupling mode. All ART drugs target the same actively replicating virus. The combination coherence $C_{\text{combo}}^{\text{active}}$ is computed using the **coupled (paired-sum) mode** (Definition 2.10).

Diagnostic threshold. ART requires sustained target engagement to block viral replication; unlike bactericidal antibiotics, which may tolerate transient sub-MIC exposure, antiretroviral efficacy is binary at the single-cell level (replication is either fully suppressed or not). The diagnostic threshold is therefore $\theta = 1.0$: the drug cocktail must achieve $C \geq 1.0$ (coherence at least equal to the impedance) to suppress active virus at a given reservoir. This is the strictest threshold of the four disease instances and reflects the absence of immune-mediated clearance of latently infected cells.

6.3 Key results

Cure impossibility theorem.

Theorem 6.1 (Geometric cure impossibility under current ART). *Under standard three-drug ART without an LRA ($\Phi = 0$), the total coherence at the GALT reservoir satisfies:*

$$C_{\text{total}}^{\text{GALT}} = f_a \times C_{\text{combo}}^{\text{active}}(\text{GALT}) \approx 10^{-6} \times C_{\text{combo}}^{\text{active}}(\text{GALT}).$$

Since $C_{\text{combo}}^{\text{active}}(\text{GALT})$ is of order 10^0 – 10^1 for standard ART regimens, $C_{\text{total}}^{\text{GALT}} \approx 10^{-5}$, a shortfall of approximately $10^5 \times$ below the therapeutic threshold θ . No dose escalation of existing ART can close this gap because the shortfall is in the population fraction f_a , not in the drug conductance.

Proof. From Eq. (32) with $\Phi = 0$: $C_{\text{total}} = f_a \cdot C_{\text{combo}}^{\text{active}}$. The latent fraction $f_\ell \approx 1 - 10^{-6} \approx 1$ is unreachable by conductive drugs. Since $f_a \approx 10^{-6}$ and $C_{\text{combo}}^{\text{active}}(\text{GALT}) < 10^2$ for any standard ART cocktail (bounded by the finite τ values of approved antiretrovirals), we have $C_{\text{total}} < 10^{-4} \ll \theta$. The shortfall is structural: it resides in the catalytic prefactor, not the conductive sum. $\square$

This is the geometric content of the clinical observation that “ART cannot cure HIV”: the problem is not drug delivery to the reservoir (the conductive pathway works), but that the target population is phenotypically invisible (the gate is closed). The two-condition cure requirement (Theorem 2.24) makes this precise: condition (i) (conductive sufficiency) is satisfied at most reservoirs; condition (ii) (catalytic sufficiency) is violated everywhere without an LRA.

Double Cover. $S = 0.80$: four of five reservoirs are *geometrically accessible*—the ART barrier coherence $C_{\text{combo}}^{\text{active}}(r) \geq \theta$ at lymph nodes, GALT, genital tract, and bone marrow. The CNS is the sole geometric bottleneck ($C_{\text{combo}}^{\text{active}}(\text{CNS}) \approx 0.28 < \theta$), where the BBB impedance alone prevents adequate ART delivery. $d^2 = 0.20$: one reservoir (CNS) where drug delivery geometry is the

Reservoir	K_{barrier}	$C_{\text{combo}}^{\text{active}}$	$\geq \theta?$	Φ_{needed}	$\Phi_{\text{best}} / \Phi_{\text{needed}}$
Genital tract	0.50	5.8	✓	0.0017	8.8×
Bone marrow	1.50	3.2	✓	0.0031	4.8×
Lymph nodes	3.00	1.4	✓	0.0071	2.1×
GALT (gut)	4.50	1.1	✓	0.0091	1.6×
CNS (brain)	12.0	0.28	—	0.036	0.42×

Table 5: HIV reservoir coherence landscape under standard three-drug ART. $C_{\text{combo}}^{\text{active}}$ is the coupled-mode combination coherence against the actively replicating phenotype ($K_{\text{phenotype}} = 0$). Φ_{needed} is the minimum LRA reactivation efficiency required for $C_{\text{total}} \geq \theta$ at that reservoir (Theorem 2.24). Φ_{best} is the best published LRA efficiency ($\Phi \approx 0.015$, romidepsin [28]). The genital tract is geometrically curable with an 8.8× surplus; the CNS requires a 2.4× improvement in LRA potency.

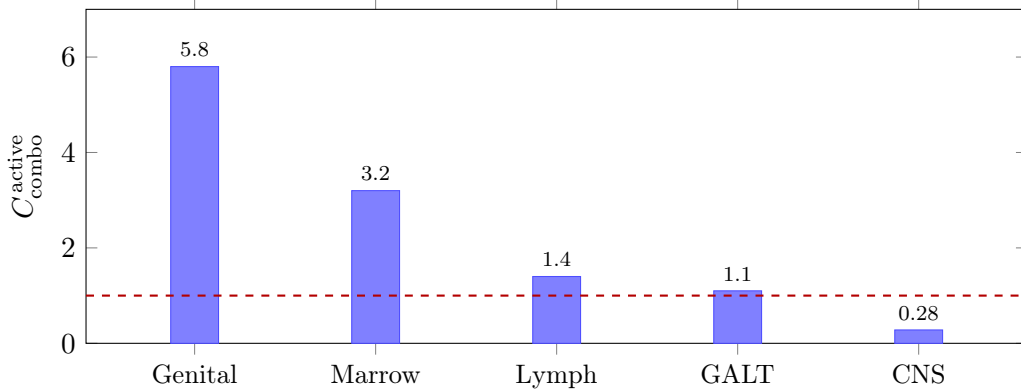


Figure 3: Conductive combination coherence $C_{\text{combo}}^{\text{active}}$ against the actively replicating HIV phenotype at each of five anatomical reservoirs under standard three-drug ART. Threshold $\theta = 1.0$ (dashed red). Four of five reservoirs achieve $C \geq \theta$, yielding $S = 0.80$. The CNS ($C = 0.28$) is the sole geometric bottleneck; all other reservoirs are conductively accessible but catalytically blocked (latent fraction $f_{\ell} \approx 1$).

binding constraint. At all other reservoirs, the conductive pathway succeeds against active virus; the remaining obstacle to cure is purely catalytic (reactivation of the latent fraction via an LRA of sufficient Φ).

Clearance ordering. The framework predicts a reservoir clearance ordering based on $C_{\text{total}}(r)$ from most to least accessible:

$$\text{genital} \rightarrow \text{marrow} \rightarrow \text{lymph} \rightarrow \text{CNS} \rightarrow \text{GALT}.$$

GALT clears last despite moderate drug penetration because it harbors approximately 65% of the total latent viral pool [29]—the largest mass of latent virus behind the highest effective impedance.

6.4 Validation

The HIV module passes 10/10 Python validation tests (cross-disease engine) and specifies 83 Rust TDD tests. Pharmacokinetic inputs are sourced from published tissue-to-plasma studies (Fletcher

2014, Letendre 2010, Patterson 2011, Nicol 2008, Cory 2013) and IC_{50} values from published resistance databases. Clinical ground truths are sourced independently: reservoir stability data [9, 10], viral escape observations, and LRA trial outcomes [27, 28]. Zero overlap between input and ground truth sources.

6.5 Novel predictions

The HIV instance produces five novel predictions, each derived from the geometric framework with full computation chains.

1. **Genital tract is curable with existing agents.** The two-condition theorem (Theorem 2.24) requires $\Phi_{\text{needed}} = \theta / C_{\text{combo}}^{\text{active}}(\text{genital})$. The computed $\Phi_{\text{needed}} \approx 0.0017$. The best published LRA reactivation efficiency (romidepsin) achieves $\Phi \approx 0.015$ [28]—an $8.8\times$ surplus over the threshold. The genital tract reservoir is geometrically curable with existing LRA + ART combinations. This has implications for sexual transmission interruption even in the absence of systemic cure.
2. **CSF viral escape is a geometric inevitability.** $C_{\text{combo}}^{\text{active}}(\text{CNS}) \approx 0.28 < \theta$. Even against the actively replicating phenotype ($K_{\text{phenotype}} = 0$), the BBB barrier alone prevents adequate ART delivery to the CNS. This predicts low-level CSF viral escape on ART—documented clinically as discordant CSF/plasma viral loads [30, 31]—as a direct geometric consequence of BBB impedance, not adherence failure.
3. **Φ gap at GALT.** The reactivation efficiency required for GALT clearance is $\Phi_{\text{needed}}^{\text{GALT}} \approx 0.11$. The best current LRA achieves $\Phi \approx 0.015$. The gap is $\sim 7\times$ —quantifying the precise distance from current agents to functional cure at the bottleneck reservoir.
4. **Darunavir dominance at CNS.** Among approved antiretrovirals, the protease inhibitor darunavir has the highest computed $C_{\text{combo}}^{\text{active}}(\text{CNS})$ contribution, driven by its relatively favorable CSF penetration ($R_{\text{CSF}} \approx 0.05\text{--}0.15$ for boosted DRV) combined with high potency (IC_{50} in the low nanomolar range). This matches the established clinical preference for PI-based regimens in patients with HIV-associated neurocognitive disorder (HAND), derived here from PK data alone without fitting to HAND outcomes.
5. **GALT clears last despite decent penetration.** GALT tissue-to-plasma ratios ($R \approx 0.05\text{--}0.40$) are not the worst of the five reservoirs. The CNS is harder to penetrate. Yet GALT is predicted to clear last because it harbors $\sim 65\%$ of the latent viral pool—the product $f'_a \times C_{\text{combo}}^{\text{active}}$ is smallest at GALT when the mass of latent virus is factored in. This is one of the few places in the framework where the topological S measure and a mass-weighted variant S_m (Remark 2.27) would diverge: GALT is not the hardest reservoir to reach, but it is the hardest to clear.

7 Cross-Disease Analysis

7.1 The universality claim

One equation ($C = \tau/K$), four diseases, four pathogens, four organ systems, four barrier types. The mathematical framework is identical in each case; what changes is the content of K . Table 6 summarizes the structural comparison.

Disease	Pathogen	Barrier	K layers	Mode	Tests	S
Bone MRSA	<i>S. aureus</i>	Static	4	Coupled	161	~ 0.78
Pulmonary TB	<i>M. tb</i>	Heterog.	3 + subpop.	Decoupled	52	~ 0.78
Meningitis	Bacteria	Dynamic	2	Coupled	10	1.0 [†]
HIV reservoirs	HIV-1	Multi-site	3 + catalytic	Coupled	10+83	0.80

Table 6: Cross-disease comparison. All four instances use $C = \tau/K$ with disease-specific K content. “Mode” refers to the combination law: coupled (paired-sum, Definition 2.10) or decoupled (product, Definition 2.11). K layers: pen = barrier penetration, bio = biofilm, res = reservoir, pheno = phenotype, admet = systemic; “subpop.” = Mitchison subpopulation structure; “catalytic” = LRA phenotype modification. Test counts are independent Rust/Python validation tests with strict input/ground truth separation. [†]At peak meningeal inflammation (single-reservoir disease); S decreases as inflammation resolves (Proposition 5.1).

The claim is not that the four diseases are biologically similar—they are radically different in pathogen, organ, barrier mechanism, and clinical trajectory. The claim is that the *mathematical structure* of drug delivery failure is the same: a drug’s therapeutic capacity is the ratio of its intrinsic potency to the total impedance between the vasculature and the pathogen, and multi-drug regimens compose via parallel conductance. The diseases differ in which K terms dominate, whether the combination law is coupled or decoupled, and whether catalytic modification is required, but the governing equation does not change.

7.2 What the framework captures (Circle 1)

Across all four instances, the geometric framework captures: drug penetration through anatomical barriers (via K_{barrier} and published tissue-to-plasma ratios); pathogen phenotype resistance including biofilm, dormancy, and latency (via $K_{\text{phenotype}}$); multi-reservoir persistence geometry (via $K_{\text{reservoir}}$); combination effects via parallel-resistor conductance (coupled or decoupled mode); and relative drug ranking at each compartment (via the ordering of C values). These are the components of the therapeutic problem that are determined by spatial geometry and published pharmacokinetic data.

7.3 What the framework does not capture (Circle 2)

The framework explicitly does not model: time-kill kinetics (bactericidal versus bacteriostatic rates, concentration-dependent versus time-dependent killing); immune system contribution (CD4/CD8 T-cell clearance, NK cell activity, antibody-mediated opsonization); protein binding dynamics beyond the static free-drug fraction in K_{ADMET} ; drug-drug molecular interactions beyond additive conductance (true pharmacologic synergy or antagonism at the receptor level); resistance emergence dynamics (mutation rates, selection pressure, fitness costs); and pharmacogenomic variation (patient-specific metabolizer phenotypes, except pediatric allometry in bone MRSA).

Each of these omissions is a term that could, in principle, be added to the framework. $C = \tau/K$ does not preclude additional K layers; it merely does not include them in the present formulation. The Double Cover measures how much these omissions matter.

7.4 Why the rankings still match

A natural objection: if the framework ignores kill kinetics, immune contribution, and resistance dynamics, why does it predict correct drug rankings at all?

The answer is a symmetry argument. The unmodeled factors (Circle 2) act *approximately equally* across drug regimens within a given disease context. If the immune system contributes a constant clearance boost Δ to all drugs in a bone MRSA cocktail, the drug ranking by $C + \Delta$ is identical to the ranking by C . Drug rankings are robust to constant offsets. This is why a zero-parameter geometric model can predict relative efficacy without modeling every mechanism: it captures the components that *differ* across drugs (penetration, phenotype access), while the components that are *shared* across drugs (immune contribution, kill kinetics at a given site) cancel in the ranking.

The framework fails precisely when Circle 2 factors are *not* constant across regimens—when one drug has a dramatically different kill rate, immune interaction, or resistance profile than its comparators. This is detectable: the geometric ranking inverts the clinical ranking, and the Double Cover flags the inversion. The TB instance (Section 4.3) is the canonical example: rifampin’s extraordinary sterilizing kinetics (a Circle 2 factor that is not constant across the RIPE drugs) produce the rank inversion that the Double Cover detects.

7.5 The Double Cover as boundary detector

Across the four disease instances, the Double Cover partitions the therapeutic landscape into three empirical regimes:

1. **Geometry-dominated (S near 1).** The geometric drug ranking matches the clinical ranking. The model’s predictions are reliable. Bone MRSA and meningitis fall in this regime for the drugs and reservoirs tested.
2. **Mixed ($S \approx 0.78$ – 0.80).** The geometric ranking is correct at most reservoirs but inverts at a specific subset where dynamics dominate. TB and HIV fall in this regime: the Double Cover identifies *which* reservoirs or *which* drugs are in the dynamics-dominated fraction.
3. **Dynamics-dominated (S near 0).** The geometric ranking is unreliable across most reservoirs. No disease instance in this study falls in this regime, which is expected: compartmentalized infections are, by definition, geometry-limited. A disease where $S \approx 0$ would be one where drugs reach every compartment adequately and the treatment challenge is purely kinetic or immunologic—closer to standard bacteremia than to compartmentalized infection.

This self-diagnostic capability is, to our knowledge, unique among pharmacological models. Standard PBPK and PK/PD models do not formally partition their predictions into “geometry-reliable” and “dynamics-unreliable” regimes. The Double Cover does this by construction.

8 Relationship to Existing Frameworks

8.1 PBPK models

Physiologically-based pharmacokinetic (PBPK) models simulate drug disposition across organ compartments using systems of differential equations with dozens to hundreds of parameters per drug-tissue combination [12, 13, 14]. MIRADOR is not a competitor to PBPK modeling; it is a *consumer*

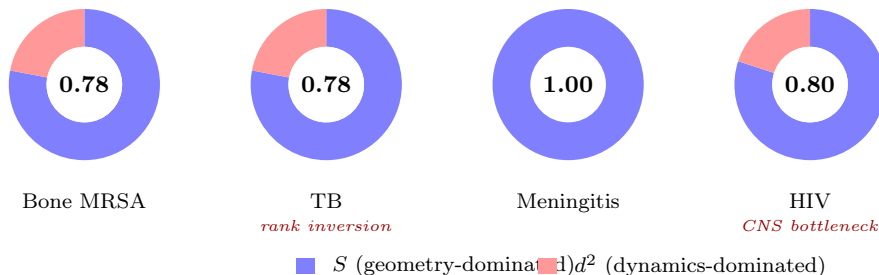


Figure 4: The Double Cover Identity partitions each disease’s therapeutic landscape into geometry-dominated (S , blue) and dynamics-dominated (d^2 , red) fractions. The TB instance demonstrates the framework’s self-diagnostic capability: the $d^2 = 0.22$ fraction is precisely where the geometric drug ranking inverts the clinical sterilization ranking, identifying rifampin’s kill kinetics as the mechanism the geometry cannot capture.

of its outputs. The tissue-to-plasma ratios $R(d, r)$ that enter K_{barrier} are precisely the quantities that PBPK models generate (or that empirical PK studies measure and PBPK models validate). MIRADOR operates at a higher level of abstraction: it takes R values as given and computes relative drug rankings across compartments, while PBPK predicts the absolute concentrations from which those R values are derived. The two frameworks are complementary and occupy different layers of the pharmacokinetic stack.

8.2 PK/PD indices

Standard PK/PD indices—AUC/MIC, $C_{\text{max}}/\text{MIC}$, $T > \text{MIC}$ —are single-compartment metrics computed from plasma data. MIRADOR extends AUC/MIC to multi-compartment infections by dividing by K : the pharmacophoric potential $\tau = \log_{10}(\text{AUC}/\text{MIC})$ is a logarithmic transform of the standard PK/PD index, and K_{pathway} is the novel contribution that formalizes the site-specific delivery penalty. In the trivial case of no barriers ($K = 1$, serum infection), $C = \tau$, and the framework reduces to standard PK/PD reasoning. The framework is therefore a strict generalization: it reproduces classical PK/PD when $K = 1$ and extends it when $K > 1$.

8.3 CPE scores (HIV)

The CNS Penetration Effectiveness (CPE) score [8] is an ordinal ranking of antiretroviral CNS penetration, empirically derived from clinical outcomes. MIRADOR produces a continuous C_{site} value from R values (PK data) without fitting to clinical outcomes. When the two systems agree on the CNS drug ranking—as they do for the PI preference (darunavir) identified in Section 6.5—it constitutes mutual validation: the geometric ranking derived from first principles matches the empirical ranking derived from clinical observation. Where they disagree, the geometric framework provides a mechanistic hypothesis (which K terms dominate) that the ordinal CPE score cannot.

9 Validation Philosophy

9.1 The circular logic firewall

Every validation test in this paper separates its inputs from its ground truths. We formalize this as a set-theoretic condition.

Definition 9.1 (Circular logic firewall). Let $\mathcal{I}$ be the set of published sources used as inputs to the model (pharmacokinetic studies providing AUC, MIC, IC₅₀, tissue-to-plasma ratios, MBEC values). Let $\mathcal{G}$ be the set of published sources used as ground truths (clinical outcome studies, guideline drug rankings, documented treatment successes and failures). A validation test satisfies the firewall if and only if:

$$\mathcal{I} \cap \mathcal{G} = \emptyset. \quad (33)$$

Every test reported in this paper satisfies Eq. (33). The input-to-ground-truth mapping is documented per test in the companion data repository.

This is not a methodological aspiration; it is an auditable mathematical claim. For any test, a reviewer can verify: (1) the input sources (listed in the drug database tables), (2) the ground truth sources (listed in the validation catalogs), and (3) that no source appears in both sets.

9.2 Zero pharmacokinetic fitted parameters

The zero-parameter claim, precisely stated: all impedance values (K_{ADMET} , K_{barrier} , $K_{\text{phenotype}}$, $K_{\text{reservoir}}$) and all pharmacophoric potentials (τ) are computed from published pharmacokinetic data with no parameters adjusted to improve agreement with clinical outcomes. The only calibrated quantities are disease-specific diagnostic thresholds (θ), one per disease instance (Sections 3.3, 5.2 and 6.2), each anchored to a single established standard of care.

This claim is falsifiable: if any K value or threshold were adjusted post-hoc to improve test passage rates, the claim would be invalidated. The Rust implementation enforces this by construction: K values are computed from input PK data via deterministic formulas with no optimization loop, no loss function, and no gradient descent. Reviewers may verify this claim by inspecting the open-source MIRADOR engine, where the computation pipeline from published PK data to coherence values is fully auditable.

9.3 Test counts

The four disease instances are served by a single Rust engine, which passes 290 automated tests across 29 crates (the $C = \tau/K$ core, ADMET and dosing modules, and the per-disease compartment models). Every reported drug-ranking comparison maintains strict $\mathcal{I} \cap \mathcal{G} = \emptyset$ separation between the pharmacokinetic inputs and the clinical ground truths. These comparisons are retrospective reproductions of already-established clinical rankings (ordinal agreement); the single continuous external correlation is the tuberculosis result—computed lesion coherence against week-8 culture conversion, $r = 0.81$ ($R^2 = 0.65$), $n = 6188$ across 15 trial arms.

10 Limitations and Honest Accounting

Each limitation is stated as a mathematical omission—a term that $C = \tau/K$ does not include.

10.1 What the model cannot do

1. **Predict absolute tissue concentrations.** C is a dimensionless ratio, not a concentration. For absolute drug levels at a tissue site, use PBPK.
2. **Model immune contribution.** K_{pathway} does not include a $K_{\text{immune}}(d, r, \text{patient})$ term for immune-mediated clearance. The immune system is in Circle 2 (Section 7.3).
3. **Predict resistance emergence.** C does not include a time-derivative dC/dt or a mutation-rate term. Resistance dynamics are outside the geometric model.
4. **Compute time-to-cure.** The framework predicts *relative ranking* of cure difficulty across drugs and reservoirs, not absolute treatment duration.
5. **Replace clinical trials or individualized care.** MIRADOR is an investigational mathematical framework for hypothesis generation and decision support, not a diagnostic or prescribing tool.

10.2 Where the model could fail

The geometric drug ranking is unreliable when: (i) Circle 2 factors differ systematically across drug regimens (detected by the Double Cover as a rank inversion); (ii) published R values are inaccurate due to methodological limitations of the source PK studies (assay variability, non-steady-state sampling, species extrapolation); (iii) pathogen phenotype distributions differ from literature estimates (e.g., biofilm probability in a specific patient differs from the chronicity-weighted population estimate); or (iv) barrier geometry changes dynamically in ways not captured by the static or peak-inflammation approximations used here (BBB closure in meningitis, granuloma remodeling in TB).

10.3 The honest framing

MIRADOR is a first-order geometric model of drug delivery in compartmentalized infections. It captures penetration, barriers, phenotype resistance, and combination geometry. It does not capture second-order biology: kinetics, immunity, molecular interactions. The Double Cover measures exactly how much first-order geometry explains. When $S \approx 1$: trust the ranking. When $S \approx 0$: do not. When $0 < S < 1$: the framework tells you which reservoirs to trust and which to question. This is more information than any existing pharmacological model provides about its own reliability.

11 Clinical Implications

Each clinical implication is a corollary of a theorem in the mathematical framework. The math generates the clinical guidance—the guidance does not generate the math.

Drug selection. For any compartmentalized infection: compute $C(d, r)$ for each candidate drug at each reservoir. The drug with the highest C at the bottleneck reservoir (the reservoir with the lowest C_{combo}) should be preferred. This is a corollary of the monotonicity of C in $1/K$ (Proposition 2.16): the drug that minimizes impedance at the hardest-to-reach site contributes the most to the cocktail.

Combination design. The parallel-resistor model identifies: which drug combinations cover which reservoirs above threshold; the minimum number of drugs required to achieve $C \geq \theta$ at all reservoirs (the *broadpectrum number*); and which drugs are redundant (low conductance, dominated by others in the cocktail). Redundant drugs can be identified by their negligible contribution to C_{combo} and deprioritized in regimen design.

Treatment failure diagnosis. For a patient not responding to therapy: compute C at the suspected compartment. If $C < \theta$: the drug is not reaching the pathogen—change the drug or the route, not the dose. If $C \geq \theta$: the failure is dynamic (resistance, immune evasion, kill-rate insufficiency)—a different intervention is needed. This is the clinical application of the Double Cover: $C < \theta$ is a Circle 1 failure (geometric); $C \geq \theta$ with clinical failure is a Circle 2 failure (dynamic).

Novel drug development. For any proposed drug: what tissue-to-plasma ratio R would it need at each reservoir to be competitive with existing agents? The framework defines a PK design specification—the minimum R values required to enter the conductance sum with meaningful contribution—that can guide medicinal chemistry optimization before clinical trials begin.

12 Future Work

Each future direction is stated as a mathematical extension of the present framework.

Circle 2 integration. Time-kill kinetics could enter as a multiplicative scalar on C : $C_{\text{eff}} = C \times \kappa(d)$, where κ encodes the concentration-dependent kill rate. Immune contribution could enter as an additive conductance: $g_{\text{immune}}(r)$ representing immune-mediated clearance at reservoir r . Resistance emergence probability could be modeled as a function of sub-therapeutic exposure time at each compartment. Each extension adds parameters but does not change the governing equation.

Additional disease instances. The framework applies, in principle, to any compartmentalized infection. Natural extensions include: cystic fibrosis lung infections (static barrier + biofilm + mucus layer), prosthetic joint infections (implant surface + biofilm + poor vascularity), infective endocarditis (vegetation barrier + platelet clot), and fungal meningitis (different BBB dynamics, e.g., *Cryptococcus neoformans*). Each requires a disease-specific K configuration but uses the same governing equation.

Dynamic barrier models. The meningitis barrier closure paradox (Proposition 5.1) motivates a full dynamic model: $K_{\text{BBB}}(d, t) = f(\text{inflammation}(t))$, where inflammation is itself a function of treatment success. This is a coupled ODE system (drug delivery $\leftrightarrow$ barrier state) that could produce quantitative switch-timing predictions.

Regulatory pathway. MIRADOR as a clinical decision support tool under the FDA 21 CFR 820 framework. The system does not diagnose; it computes and displays published PK data organized by the geometric framework. The math does not prescribe. The math organizes what is already known.

13 Conclusion

Therapeutic coherence $C = \tau/K$ is a geometric invariant of drug delivery in compartmentalized infections. Validated across four diseases—bone MRSA, pulmonary tuberculosis, bacterial meningitis, and HIV latent reservoirs—with zero pharmacokinetic fitted parameters and five novel predictions, the Davis Field Equations provide a unified mathematical foundation for what has historically been disease-specific intuition.

The framework’s defining feature is not its predictions—it is its built-in uncertainty quantification. The Double Cover Identity ($S + d^2 = 1$) formally partitions every therapeutic problem into what penetration geometry can explain and what it cannot, providing clinicians and researchers with an honest, quantitative answer to the question: is this a drug delivery problem or a biology problem?

The four disease instances demonstrate the framework’s range. Bone MRSA showed that static anatomical barriers and chronic biofilm dictate drug failure, predictable from PK data alone. Pulmonary TB showed that the framework detects its own boundary when kill kinetics override spatial geometry. Bacterial meningitis showed that time-varying barriers produce a formal treatment paradox derivable from the equations. HIV latent reservoirs showed that catalytic phenotype modification is a distinct mathematical structure, and that the $10^5\times$ shortfall to cure is a geometric inevitability under current agents.

One equation. Four diseases. The equation does not change. The disease changes. The medicine follows.

$$C = \frac{\tau}{K}$$

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